

HOPE Clinical Operational Manual:
A Clinician's Guide

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CONFIDENTIAL

NOTICE

The Clinical Operational manual was developed to provide staff with the fundamental guidelines of the processes and practices of delivering forensic services to our clientele and working with our stakeholders.

The information provided in this document is confidential and proprietary to HOPE Program and may not be used for any purpose other than to inform Probation and Parole agencies of the operations of the program and to guide the delivery of sex offender specific services. This document and the information contained in it must not be disclosed to any person other than an employee or advisor of the Probation or Parole agency. In the event of such disclosure, the parties receiving this document should be notified that it contains HOPE Program's confidential and proprietary information and must be kept confidential.

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NOTICE: If any part of this manual is later found to be incorrect or inapplicable, it does not invalidate any other section of it. Please notify the Executive Director of Clinical Operations, immediately, if you have any questions about any part of this manual or its contents. Updates will occur as laws change, programs develop, and HOPE grows.

TREATMENT AND MANAGEMENT PHILOSOPHY

HOPE Program was founded in September of 2008 by Shannon Smith, LCSW, who has dedicated her professional career to helping individuals convicted of a sexual offense, or suffering from emotional, behavioral, or psychological problems. Based on the extensive experience and insights she gained over the years, Ms. Smith has developed a unique vision for a clinical program that addresses client's individual needs and risk level.

HOPE Program utilizes the latest best-practice research to provide services to individuals and families in order to reduce their risk of reoffending or engaging in problematic sexual behaviors. The primary goal of HOPE is to combine the latest research with practical psychotherapy techniques to provide effective, personalized, and affordable care that leads to reduced recidivism and healthier lifestyles. HOPE Program maintains strict adherence to the Association for the Treatment and Prevention of Sexual Abusers (ATSA) and California Coalition on Sexual Offending (CCOSO) guidelines and best practices, and is governed by the California Sex Offender Management Board (CASOMB).

HOPE's Core Focus is Forensic Mental Health Treatment and our Core Values are:

- Passionate about creating better lives
- Committed to victim prevention
- Approachable and warm
- Collaborative, respectful, motivated, and driven
- Acting with integrity
- Compassionate, conscientious, thoughtful, and professional

We strive to deliver our services according to the following guiding principles:

- Highly personalized service
- Experience and professionalism

- Adherence to implementation of empirical-based and state-of-the art research for sex offenders
- Adherence to best practices and treatment standards, as well as steadfast ethical standards
- Serving local community

The key characteristics of our Program are as follows:

Research-based Treatment

- Strict adherence to the Containment Model while incorporating evidence-based offender rehabilitation models (i.e., Risk Need Responsivity, Good Lives Model) and strength-based approaches
- Flexible curriculum and recovery plan tailored to individual needs and risk factors of the clients
- Graduation criteria aligned to goal achievement and reduction of risk to reoffend

Specialized Psychotherapy

- Experienced CASOMB Certified practitioners trained in sex offender-specific treatment, psychosexual evaluations (including specialized risk assessments) and personalized case management techniques
- Identification of client needs based on structured assessment protocols and clinical diagnoses obtained during the assessment process
- Goal-based approach to treatment of sexual offenders
- Adaptive case management based on continuous progress monitoring and frequent assessments
- Clinical staff are members of ATSA and/or CCOSO and professional organizations providing ethical guidelines and treatment standards for the industry

Efficient Operating Model

- HOPE uses a treatment approach such that all clinical staff are able to provide services and contribute to the treatment of any client using TreatAnyone (TA), a data management platform that supports treatment and improves continuity of care. Stakeholders are given access to TA to assist in maintaining transparency, effective communication, and better oversight, which provides a closer and more direct collaboration between therapists and supervising officers.

CORE FOCUS: FORENSIC MENTAL HEALTH TREATMENT



CORE VALUES

- Passionate about creating better lives
- Committed to victim prevention
- Approachable and warm
- Collaborative, respectful, motivated, and driven
- Acting with integrity
- Compassionate, conscientious, thoughtful, and professional

HOPE PROGRAMS CODE OF PRACTICE FOR SEX OFFENDER THERAPISTS

In order to achieve our Core Focus and abide by our Core Values, HOPE operates within The Code of Practice commonly known as The Code of Ethics for sex offender therapists, adapted from The Association for the Treatment and Prevention of Sexual Abusers (ATSA; https://www.atsa.com/). ATSA has published Practice Standards and Guidelines, which include a section on Professional Conduct.

The Code of Practice presents a framework for making professional decisions and augmenting our moral compass. The strength of HOPE ultimately resides in the commitment of its clinical team to common goals, beliefs, and values.

It is the responsibility of HOPE and its employees to ensure we are dedicated to supporting values of basic human dignity and respect, as well as common goals related to community safety and preventing sexual violence. Employees are responsible for familiarizing themselves with the ATSA Adult Practice Guidelines, the ATSA Adolescent Practice Guidelines, and this Code of Ethics, as seen below. For additional information on ATSA guidelines, visit here: https://members.atsa.com/learn/Details/guidelines-professional-code-of-ethic-194332.

General guidelines	• The code supplements any other ethical or professional code practiced by individuals. • Therapists should always adhere to the highest professional standards and should maintain and develop their skills. • Therapists agree to work within the treatment model of the program.
Competence	• Therapists should not practice beyond their level of competence and training. • Facilitators should always respect the need for treatment integrity. • Facilitators should take responsibility for recognizing the limits of their competence.
Conduct	• Treatment should never will not be conducted in a spirit of retribution for offending. • Facilitators should will not compromise their professionalism with violence, racism, derogatory language, sexual contact with clients or their families, or improper receipt of gifts. • Facilitators should will only deliver treatment if in a fit mental, emotional, and physical state. • Facilitators should will model appropriate respectful behavior in their relationships with colleagues and with clients
Confidentiality	• Therapists should will treat all information from their clients with appropriate professional discretion. • Therapists must be clear with clients about the conditions under which information about them will be shared outside treatment context. • All materials relating to clients must be stored securely. • False assurances of confidentiality must not be given to clients.
Consent	• The purpose and implications of all treatment exercises must be explained to clients at the outset. Clients must never be misled about the purposes of an exercise or procedure. • Therapists must take steps to ensure that clients do not possess false expectations about the benefits of treatment. • Clients who wish to withdraw from the program must be treated with sensitivity.

Colleagues	• Therapists must bring any concerns about colleagues to the person concerned or to a manager without delay. • Therapists must take all steps possible to communicate and work constructively with other professionals involved with the client. • Therapists should respect each other for the unique contribution that each person brings to sex offender treatment.

SAFETY PROTOCOLS

Safety is of upmost importance, and although we have been providing services in a community setting for several years without incident, it is important to be mindful of the clientele we serve, maintain healthy boundaries, and take measures to reduce risk as much as possible.

To ensure the safety of our employees and clients:

- Do not keep any secrets with a client at any time.
- Do not offer to hire or provide any money to a client at any time.
- Do not give or accept gifts to/from clients.
- All services are to be conducted during the business hours of the agency. Any service outside of standard operating hours, requires prior written authorization from the Executive Clinical Director.
- After the conclusion of group, ensure all members of the group leave the building together. In the evenings, no clinician should ever stay behind and alone with a client when no other people are nearby.
- All communication with clientele should take place in a HOPE office. Do not engage in conversations with a client outside of the office. For example, do not have conversations with clients in the hallway, on the sidewalk, or in any environment that is outside of the clinic.
- All interactions with clientele are to remain professional and appropriate at all times. Clinicians will not engage in derogatory or vulgar language.
- All social media connections and/or sharing personal contact information with clients is strictly prohibited. This includes, but is not limited to:
    a. Do not friend any clients on Facebook or any other social media applications.

b. Do not share, follow, or engage in any form of social media with clients.

a. Do not share any personal photos or videos with clients at any time.

b. Do not take a picture with a client or allow a client to take a picture of you.

c. Do not hug or touch a client.

d. Report any and all suspicious, flirtatious, or concerning behavior to your supervisor and the clinical director on the same day the incident occurred.

e. Document any and all suspicious, flirtatious, or concerning behavior in the client's file on the same day the incident occurred.

f. Do not share your personal contact information, such as your home or cell phone number. Only communicate via email or telephone through your work provided communications. All client communications outside of the therapy room are to be documented in the client's file in TA.

g. All email communications received from clients must be saved in their client file; clinicians do not provide therapy via electronic communication (e.g., email or text).

h. Texting clients is prohibited.

- If you have any questions about your safety with any client in any context, end the session or your contact with the client. Excuse the client from the agency and immediately contact your supervisor.
- In any situation where there is a concern for your physical safety or that of others, immediately end the group or the individual session and call 911.
- All clients are to walk in front of staff members, as to avoid clients being out of an employee's line of sight.
- Employees and clients are to take separate elevators when entering and exiting the building.
- During all sessions, clinicians are to sit by the door to ensure access to an exit is not blocked, should a client turn violent and the clinician needs to escape.
- In clinics where you are working with other employees during normal operating hours, notify your colleagues that you are leaving for lunch, taking a break outside of the office, or when you leave the office for the day.

- Clients who bring book bags (or any type of bag) to treatment are to store them in a designated area of the treatment room, away from their person. They can collect the book bag at the end of the session.
- Clients are not allowed to take book bags into the restroom in order to prevent theft, drug use, or consumption of alcohol.
- Clients may only go to the bathroom by themselves and not in pairs or in a group, to prevent theft, drug use, conflict, or consumption of alcohol.

To view the full list of practices our teams need to follow in order to promote a safe work environment, visit HOPE's drive or review our Safety Procedure's Binder here: https://drive.google.com/file/d/1hHt3oOZoalLAG558aqq28OGL-YBzY80L/view?usp=sharing

OVERFAMILIARITY

Familiarity or overfamiliarity with our clients must be closely monitored to ensure it does not happen. Overfamiliarity means engaging in conversations or any type of communication that includes personal topics. No employee may engage in familiarity with clientele who are referred to the HOPE Program. Conversations shall be limited to work and treatment related topics only. The behaviors listed below may place an employee at risk of developing an inappropriate relationship with an offender:

- Looking forward to seeing a particular client when you come to work.
- Doing things with a client you would not want your family or supervisor to know about.
- Being reluctant to have a co-worker observe your behavior for an entire day.
- Talking about personal matters with client.
- Asking a client to do personal favors for you.
- Receiving personal advice from a client.
- Saying anything to a client you would not want tape recorded.
- Having fantasies of touching a client.
- Believing that you have the right to touch a client.
- Looking forward to sharing good or bad news with a particular client.
- Thinking clients are not allowed to say "no" to you, no matter what you ask.
- Allowing clients to talk about sexual experiences or sexual fantasies, beyond what is treatment appropriate.

- Allowing clients to talk about or tell sexual jokes in your presence.
- Disclosing unauthorized information learned from the supervising officer or agent, or by way of a CTM.

For more information, please see the Boundaries and Overfamiliarity training video here https://drive.google.com/file/d/1Eg5a99fDk4nEte_GWWErXycJBpvyu7DN/view?usp=sharing

SUICIDAL CLIENTS

Clients may become or present as suicidal or dangerous to others while engaged in treatment at HOPE Program. As we have a duty of care to our clients, we must take all precautions to assess the level of risk to the client, which then informs subsequent responses.

To assess safety, it is the expectation that all HOPE Program staff and clinicians in training

- Assess risk of suicide and/or danger to others at initial intake and for the duration of HOPE treatment
- Provide, as a priority, prompt support to address the immediate safety needs of a client presenting at any level of risk of suicide and/or danger to others

POLICIES:

Purpose and Scope

This policy provides guidance for HOPE Program staff and clinicians in training in responding to client observation, allegation, and disclosures indicating potential suicide and/or danger to others. This policy applies to all programs and services operated by HOPE Program and all staff and clinicians in training involved in client support and services provision. This policy does not prescribe client treatment planning, intervention goals and strategies, or specific counseling techniques and theories.

Definitions

a. Risk factors – factors such as biological, physiological, social, and cultural agents that are associated with suicide/suicide ideation and increase their probability
b. Suicide prevention – actions or initiatives to reduce the risk of suicide among populations or specific target groups

c. Self-harm – deliberate damage to one's body without the intent to die (also referred to as self-injury)
d. Danger to others - act, attempt, or threat of substantial and imminent harm to another
e. Suicidal ideation – thoughts about attempting or completing suicide
f. Suicidal behavior - includes the spectrum of activities related to suicide and self-harm including suicidal thinking, self-harming behaviors not aimed at causing death, and suicide attempts
g. Suicide attempt - a potentially self-injurious act intended to end one's life, but does not result in death
h. Suicide – the act of purposely ending one's life
i. Postvention – interventions to support and assist the bereaved after a suicide has occurred

Principles

HOPE Program is committed to good practice in the prevention of suicide and harm to others through the development, implementation, and review of policies and procedures based on current evidence. When responding to suicidal behavior and those dangerous to others, the physical and emotional safety of the client, staff, and clinicians in training is paramount. Thus, HOPE Program ensures that staff and clinicians in training receive a level of training and supervision appropriate to their role in responding to clients at risk of suicide and or dangerous to others.

Risk Management

HOPE Program acknowledges, and is prepared for, clients disclosing suicide or thoughts of harming others through a range of avenues including disclosure to staff, clinicians in training, and/or other clients. Assessment and responses to the risk of suicide and those posing as dangerous to others are undertaken by staff who are appropriately trained and qualified, using evidence-based assessment and response practices. All indications of suicide and danger to others are taken seriously. A first aid kit is available in accessible areas. All staff and clinicians in training are supported to recognize the limits of individual roles and competencies and actively facilitate links to further levels of care where necessary.

Outcomes

HOPE Program identifies and responds to clients at risk of suicide or a danger to others. Staff and clinicians in training receive adequate training and supervision in responding to clients either at risk of suicide or posing a danger to others.

Related Legislation

Emergency Detention Under Welfare and Institutions Code Section 5150

5150 is the number of the section of the Welfare and Institutions Code, which allows a person with a mental illness to be involuntarily detained for psychiatric hospitalization. A person on a 5150 can be held in the psychiatric hospital against their will for up to 72 hours. There needs to be a **probable cause** to take any person into custody and to place such a person in a county-designated facility for the initial 72-hour treatment and evaluation (i.e. "5150").

Probable Cause

Probable cause is defined as facts known to the authorized person that would lead a person of ordinary care and prudence to believe, or to entertain a strong suspicion, that the person detained is **mentally disordered and** is a **danger to self or others**, or is **gravely disabled**. In other words, in order to be detained under W& I Section 5150, the person must be, **"as a result of a mental disorder," a danger to self or others or gravely disabled.**

Danger to self

This criterion may be either deliberate intention to injure oneself (i.e. overdose) or a disregard of personal safety to the point where injury is imminent (i.e. wandering about in heavy traffic). **The danger must be present, immediate, substantial, physical, and demonstrable.**

Examples could include some or all of the following:

- Words or actions showing intent to commit suicide or bodily harm
- Words or actions indicating gross disregard for personal safety
- Words or actions indicating a specific plan to suicide
- Means available to carry out suicide plan (i.e. pills, firearms present or available)

Danger to others

This criterion should be based on words or actions that indicate the person in question intends to cause harm to a particular individual or intends to engage in dangerous acts with gross disregard for the safety of others.

Examples could include some or all of the following:

- Threats against particular individuals
- Attempts to harm certain individuals
- Means available to carry out threats or to repeat attempts (i.e. firearms, or other weapons)
- Expressed intention or attempts to engage in dangerous activity

Professional Development and Training

Staff and clinicians in training receive training in suicide risk assessment and interventions appropriate to their role.

Administrative staff receive mandatory introductory training to assist in identifying warning signs and for referring clients to appropriate staff members. HOPE Program Directors ensure staff and clinicians in training attend training programs delivered by providers with an appropriate level of expertise.

HOPE Program recognizes that suicide risk assessments and interventions are core skills for staff with direct client contact, and that knowledge and practice is reviewed and updated regularly.

Professional Supervision and Support

HOPE Program recognizes that supporting clients who are at risk of suicide and/or self-harm can be challenging and demanding. HOPE Program is committed to monitoring and attending to the impact of client suicide and self-harm on staff and clinicians in training.

Supervision

Staff and clinicians in training notify their supervisor or HOPE Program Director once a client suicide or self-harm risk is identified to develop immediate and longer-term risk management

planning. Staff and clinicians in training are provided with internal supervision and debriefing to address the emotional impacts that may arise from working with clients in distress.

Policy and Practice Review and Evaluation

HOPE Program reviews its policies and procedures based on feedback from staff and clinicians in training, as well as results of service evaluations and new or emerging evidence in suicide and self-harm prevention and management.

Policy Implementation

This policy is part of all HOPE Program staff and clinicians in training orientation processes and all are responsible for understanding and adhering to this policy. This policy should be referenced in relevant HOPE Program policies, procedures, and other supporting documents to ensure that it is familiar to all program staff and clinicians in training and actively used.

PROCEDURES:

Assessment and Interventions

As part of initial intake assessment and ongoing treatment work, it is important that all HOPE Program clients are assessed for risk of suicide and danger to others should there be any indication that such risk exists. In conducting client suicide and danger to others risk assessment, cultural considerations are addressed as part of the assessment process.

If staff or clinicians in training are unsure about a client's suicide or danger to others risk, they ensure the client's physical safety before consulting with their supervisor, senior colleague, or a peer. If a client is assessed as being at risk of suicide and/or danger to others, intervention strategies to decrease the risk are developed and implemented.

Both objective and subjective evidence is used to determine a client's risk of suicide and danger to others. A comprehensive assessment draws on all available information including: interviews with the client, observation, medical, psychiatric, and personal history, feedback from other staff, and information from the client's support system.

SAFE-T

HOPE Program utilizes Suicide Assessment Five-step Evaluation and Triage (SAFE-T) process to assess risk of suicide and to intervene appropriately.

IDENTIFY RISK FACTORS

Note those that can be modified to reduce risk

⇩

IDENTIFY PROTECTIVE FACTORS

Note those that can be enhanced

⇩

CONDUCT SUICIDE INQUIRY

Suicidal thoughts, plans, behavior, and intent

DETERMINE RISK LEVEL /INTERVENTION

Determine risk. Choose appropriate intervention to address and reduce risk

⇩

DOCUMENT

Assessment of risk, rationale, intervention, and follow-up

Suicide Inquiry

Suicide inquiry involves asking specific **DIRECT** questions about thoughts, plans, behaviors, and intent. See specific questions to ask here (https://docs.google.com/document/d/0B2QQX0O1SdZbLTZfVE51NWtXYlB1YzQwcWxDRHpNNllsNFY4/edit?resourcekey=0-UBTmFd54O8-6DRJTfvp-SA) with an overview below:

- Ideation: Frequency, intensity, duration—in last 48 hours, past month, and worst ever.
- Plan: Timing, location, lethality, availability, preparatory acts.

- Behaviors: Past attempts, aborted attempts, rehearsals (tying noose, loading gun) vs. non-suicidal self-injurious actions
- Intent: Extent to which the patient (1) expects to carry out the plan and (2) believes the plan/act to be lethal vs. self-injurious. Explore ambivalence: reasons to die vs. reasons to live.

For Youths: Ask parent/guardian about evidence of suicidal thoughts, plans, or behaviors, and changes in mood, behaviors, or disposition.

Homicide Inquiry: When indicated, especially in character disordered or paranoid males dealing with loss or humiliation, inquire on four bulleted areas listed above, using the link for direct inquiries.

Risk level / Intervention

Where a client is thought to be suicidal, HOPE Program staff and clinicians in training should make themselves available for brief consultation at any time within office hours. Decisions as to how to safeguard the client should be made whenever possible with the client's active participation in the decision-making process. Available options should be discussed (e.g. additional treatment sessions, phone contact with the provider within office hours, voluntary hospital admission, referral to medical or psychiatric services, out of hours contact with area mental health crisis team where appropriate, and with the client's consent, consultation with the client's family).

If risk of suicide is confirmed, HOPE Program staff and clinicians in training are guided to perform a combination of the following activities according to the risk rating:

- Monitor and staff the client regularly
- Identify client's potential supports
- Contract with client to seek immediate assistance if fleeting thoughts become more serious or depression deepens
- Request permission to contact support system
- Ensure that the client is not left alone

- Call 911 to initiate 5150
- Consult with a colleague or supervisor for guidance and support

Assessment of risk level is based on clinical judgment. Clinicians and staff in training are to reassess as client or environmental circumstances change. The chart below is intended to represent a range of risk levels and interventions, not actual determinations.

RISK LEVEL	RISK/PROTECTIVE FACTOR	SUICIDALITY	POSSIBLE INTERVENTIONS
High	Psychiatric diagnoses with severe symptoms or acute precipitating event; protective factors not relevant	Potentially lethal suicide attempt or persistent ideation with strong intent or suicide rehearsal	Call 911 to initiate "5150." Admission generally indicated unless a significant change reduces risk. Suicide precautions (e.g. the client is not to be left unsupervised)
Moderate	Multiple risk factors, few protective factors	Suicidal ideation with plan, but no intent or behavior	Admission may be necessary depending on risk factors. Develop crisis plan. Give emergency/crisis numbers. A No-Harm Contract should be entered.
Low	Modifiable risk factors, identifiable strong protective factors.	Thoughts of death, no plan, intent, or behavior	Outpatient referral (i.e. medication evaluation), symptom reduction. Give emergency/crisis numbers. Client is to be monitored frequently. Staff and clinicians in training assist client in identifying potential supports. A No-Harm Contract should be entered into.

Documentation and Collateral Contact

Client file documentation (i.e. assessments, progress notes, collateral notes, Status Alert, Treatment Plan, Monthly Treatment Report, Crisis Treatment Plan) is comprehensive and accurate and details risk level and rationale; treatment plan to address/reduce current risk (e.g.,

medication, setting, psychotherapy, contact with significant others and supervising officers/agents, consultation); follow-up plan. For youths, treatment plan should include roles for parent/guardian.

Client documentation is completed by the end of the business day. If a client is assessed as being at risk of suicide or self-harm, HOPE Program staff or clinicians in training notify their supervisor to jointly develop a plan for further assessment and intervention.

Immediate safety concerns are addressed before developing longer term risk management plans. HOPE Program staff and clinicians in training contact client's supervising officer by phone (use mobile phone numbers to reach officers and agents when in the field). This is followed up by sending Status Alert regarding the incident. These actions are followed by progress and collateral notes in the client's file.

No-Harm Contract

Client No-Harm contracts provide a written and verbal agreement for the client not to engage in suicidal or self-harming behavior. A client No-Harm contract depends on an established clinician-client relationship and capacity of the client to provide informed consent.

A written No-Harm Contract template is used by HOPE Program where the client is able to provide informed consent. The No-Harm Contract includes:

- A guarantee not to harm themselves (should have a specific time frame)
- Assisting the client in suicide proofing their environment (i.e., getting rid of potentially lethal doses of medications, weapons, etc.)
- Actions that client can implement if suicidal thoughts return
- Identifying referrals client can use to respond to triggers for suicidality (e.g. suicide hotline phone number, HOPE Program clinician contact info, etc.)
- Support structure that they can use as a resource (e.g. friends, family)
- List of crisis services
- Appointment day/time for the client to check in with HOPE Program clinician on the phone or in person between the regularly scheduled sessions

You can find a copy of HOPE's No-Harm Contract here:
https://docs.google.com/document/d/0BwJx1f27HdYtT3dKR21hc2RzNk0/edit?resourcekey=0-2VnDaWKDf-SSCfaEeYuGCw

Developing a Safety Plan

Once an immediate suicide and/or self-harm crisis has passed, clients are assisted in treatment to develop a safety plan which identifies a particular course of action they can follow if they are at risk of suicidal or self-harming behavior in the future. The safety plan should include details regarding the client's support contacts such as family, friends, and available community resources. Clients are actively involved in decision making processes about how to protect their safety and prevent suicide or self-harm.

You can find a copy of HOPE's Safety Plan here:
https://docs.google.com/document/d/1wp65sLs3wfff1WlHY80wGBcHS9kpyg-r/edit

Maintaining Client Contact

A client who has been assessed as being at risk for suicide and/or self-harm and does not attend their scheduled appointments will need to be actively followed up by the clinician to ascertain safety needs.

Relevant staff members are informed of specific client arrangements where necessary. For example, Office Manager (OM) is instructed to give priority to particular client calls.

Confidentiality

HOPE Program has a duty of care to do everything reasonable to prevent a client's suicide or self-harm.

The right of confidentiality is not absolute and is balanced against duty of care. HOPE Program has a legal and ethical responsibility to disclose information where not reporting might contribute to harm to a client or another person.

Clients are advised of the limits of confidentiality during intake assessment and if a suicide and/or self-harm situation arises. Reasonable steps are taken to obtain client agreement for information to be disclosed to a third party.

Confidential information is disclosed to those in a position to assist in client safety and the information provided is restricted to that which elicits assistance.

Suicide Postvention

HOPE Program recognizes that a client's suicide may impact other clients, staff, clinicians in training, the client's family and friends, and the wider community. Staff and clinicians in training are provided with written information and resources about suicide bereavement where appropriate.

a. **Initial Actions for Staff**

Following the death of a client from suicide or an emergency incident involving a client who is suicidal or self-harming, staff and clinicians in training are offered access to immediate debriefing support. HOPE Program nominated staff provide staff and clinicians in training with a factual briefing concerning the circumstances of the death.

Group and/or individual debriefing for staff and clinicians in training will be provided during group supervision, individual supervision, as well as during weekly clinical case consultations (i.e., staff meetings) if needed.

Staff and clinicians in training are encouraged to remain aware of their own emotional reactions and seek support from their supervisor, colleagues and other avenues as required.

b. **Initial Actions for Clients**

Following the death of a client from suicide, the individual and group treatment providers provide other current clients with relevant information about the death. The emphasis is on developing understanding without condemning or glorifying the suicidal event or client.

Following the death of a client from suicide, HOPE Program identifies clients who may be at increased risk of suicide and/or self-harm and assist them to access appropriate support.

Procedure Overview

1. If a client indicates or if HOPE Program staff or clinicians in training suspect that the client might be considering suicide or presents as a danger to others, HOPE Program staff or clinicians in training conduct a risk assessment (see the Risk Assessment form here: https://docs.google.com/document/d/0B2QQX0O1SdZbLTZfVE51NWtXYlB1YzQwcWxDRHpNNllsNFY4/edit?resourcekey=0-UBTmFd54O8-6DRJTfvp-SA

2. HOPE Program staff or clinicians in training determine the level of risk (i.e. low, moderate, high). An overarching principle must be that if a member of staff is in doubt about a person's potential for suicide or danger to others, then they should ensure that the person is safe and then consult with a supervisor, a senior colleague, or peer.

    a. **If the risk is low**, HOPE Program staff or clinicians in training perform the following:

        i. Provide emergency/crisis numbers (see Tab 12 for suicide resources in the Safety Procedure Binder on the drive)
        ii. Assist the client in identifying potential supports (e.g. family, prosocial friends, professionals)
        iii. Send a Status Alert to the supervising officer (i.e. Probation Officer, Parole Agent) for awareness and to collaborate care
        iv. Develop a Crisis Treatment Plan that includes:
            - the role and responsibilities of HOPE Program clinician
            - the role and responsibilities of the client
            - a No-Harm Contract should be signed by the client, which can be located here: https://docs.google.com/document/d/0BwJx1f27HdYtT3dKR21hc2RzNk0/edit?resourcekey=0-2VnDaWKDf-SSCfaEeYuGCw

- complete a Safety Plan to include an action plan in relation to the triggers for suicidality (e.g. attend counseling, medication). HOPE's Safety plan is located here: https://docs.google.com/document/d/1wp65sLs3wfff1WlHY80wGBcHS9kpyg-r/edit

v. Monitor the client frequently by assessing their risk on an ongoing basis (i.e. follow up by phone between sessions, schedule additional appointments, etc.).

b. **If the risk is moderate, hospitalization (i.e. "5150) may be necessary depending on risk factors.** HOPE Program staff or clinicians in training perform the following:

i. Provide emergency/crisis numbers (see Tab 12 for suicide resources in the Safety Procedure Binder on the drive)
ii. Assist the client in identifying potential supports (e.g. family, prosocial friends, professionals)
iii. Send a Status Alert to the supervising officer (i.e. Probation Officer, Parole Agent) for awareness and to collaborate care
iv. Contact the client's supervising Probation Officer (PO) or Parole Agent of Record (AOR) by phone or email and request additional follow-up (e.g. appointments, PO or AOR home visits, phone check-ins).
v. Develop a Crisis Treatment Plan that includes:
   - the role and responsibilities of the HOPE Program clinician
   - the role and responsibilities of the client
   - a No-Harm Contract should be signed by the client, which is located here: https://docs.google.com/document/d/0BwJx1f27HdYtT3dKR21hc2RzNk0/edit?resourcekey=0-2VnDaWKDf-SSCfaEeYuGCw
   - complete a Safety Plan to include an action plan in relation to the triggers for suicidality (e.g. attend counseling, medication). HOPE's Safety plan is located here:

https://docs.google.com/document/d/1wp65sLs3wfff1WlHY80wGBcHS9kpyg-r/edit

vi. Monitor the client frequently by assessing their risk on an ongoing basis (i.e. follow up by phone between sessions, schedule additional appointments, etc.).

c. **If the risk is high, clients are not to be left alone at any point.** HOPE Program performs the following:

i. HOPE Program staff or clinicians in training will call the Office Manager or, if the Office Manager is unavailable, other available HOPE Program clinician from their office to request assistance.

ii. Once the client is safe and assisted by another HOPE Program staff or clinician, the treating clinician makes a 911 call to initiate a "5150" for the mental health emergency.

iii. While waiting for the mental health emergency response (i.e. Police or ambulance) team, the client is assisted and supported by the HOPE Program clinician.

iv. HOPE Program staff or clinicians in training contact the supervising officer by phone, immediately after the client has been taken in for evaluation at a designated mental health crisis facility. The call needs to be followed up with a Status Alert to the supervising officer, unit supervisor, and assistant unit supervisor.

v. HOPE Program staff or clinicians in training complete documentation (i.e. progress notes, collateral notes, crisis treatment plan, incident report if emergency services are called).

vi. HOPE Program staff or clinicians in training follow up by frequently monitoring the client in person during scheduled appointments and/or by phone between the scheduled appointments.

vii. Other HOPE Program staff should be advised where appropriate to facilitate these options (e.g. the reception staff should be informed of client's calls which are to receive priority attention).

viii. HOPE Program staff or clinicians in training always consult their supervisors as soon as possible or, in his/her absence, an appropriate or designated colleague.

ix. Clients that have been assessed to be at risk for suicide or in danger to others should be presented for frequent review in the weekly clinical case conference meetings and monthly Containment Meetings.

COMPLIANCE WITH CASOMB AND SARATSO REQUIREMENTS

HOPE Program adheres to the California Sex Offender Management Board (CASOMB) and State Authorized Risk Assessment Tools for Sex Offenders (SARATSO) requirements and will ensure ongoing compliance by conducting internal quarterly audits. Clinicians are responsible for submitting certifications of specialized training to Human Resources. These certificates will be kept in staff's Human Resources (HR) file. Clinicians are responsible for maintaining CASOMB certification while employed at HOPE. For more information, please visit CASOMB - CA Sex Offender Management Board (https://casomb.org/), SARATSO - Home (https://saratso.org/), and HOPE Programs Frequently Asked CASOMB Questions here: https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/training/casomb

USE OF CASOMB CERTIFIED CLINICAL STAFF

The HOPE Program's clinical staff are all licensed by the State of California or license eligible. All staff are supervised by a Licensed Mental Health Practitioner (LMHP), the Clinical Director (CD), or the Executive CD. All HOPE staff participate, at a minimum, in weekly face-to-face team meetings, and supervision and training is provided. Clinical staff will be certified by CASOMB and are encouraged to become a member of Association of Treatment of Sex Abusers (ATSA; https://www.atsa.com/).

All new clinicians are required to undergo intensive sex offender specific training during their onboarding training process. During this time, they are expected to shadow and then cofacilitate with other CASOMB certified practitioners. This intensive training will ensure that all new

employees are eligible for CASOMB Certification prior to sitting with clients. Staff may be required to attend SARATSO authorized trainings or other trainings that meet the CASOMB/SARATSO criteria, not offered at HOPE. All staff are required to maintain their CASOMB/SARATSO certifications.

SUPERVISION AND PROFESSIONAL DEVELOPMENT

HOPE Program provides multiple trainings, group supervisions, and individual supervision on a weekly basis, to ensure staff are given the necessary tools to deliver effective, evidence-based services. These trainings will also support staff's professional development. In addition to 5.5 hours of weekly training (didactic, group supervision and individual supervision), there is a 1.5-hour clinical meeting where clinical staff consult on cases to ensure best practices, minimize splitting, and create effective interventions and case planning recommendations. These Clinical Case Consultation meetings are open to parole/probation and the polygraph examiner. A copy of HOPE Program's clinical supervision contracts for Associate and Apprentice level CASOMB certified clinicians can be found in TriNet at https://www.trinet.com/.

HOPE Program requires all staff be active in their role of professional development. Staff are asked to present professional trainings at HOPE Program throughout their tenure covering sex offender specific topics to ensure continued professional development. Staff are also encouraged to attend professional development conferences, specializing in sex offender assessment, treatment, and research, such as The Association for the Treatment and Prevention of Sexual Abuse (ATSA; https://www.atsa.com/). When staff participate in professional development paid for by HOPE, while at HOPE, they are required to take this training back to their team and present on the information they learned. Further, students who are training with HOPE Program are encouraged to create and present at least one didactic presentation during the training year as a way to foster their professional development.

CLINICAL SUPERVISION

All unlicensed clinicians at HOPE are required to participate in clinical supervision. For the supervisee to benefit from supervision, the expectation is that the clinician come prepared with a clinical agenda. The role of the clinical supervisor is defined as providing ongoing clinical and

operational support, guidance, education, consultation, and oversight of the supervisee's work. To this degree, the clinical supervisor is responsible, at least in part, for providing mentorship, the development of skills and growth in supervisees, and for protecting the welfare of the clients and community we serve. Supervisors help supervisees to understand their clients, their cases, and the allegations against them. Supervisors also help supervisees identify their clients' criminogenic needs, formulate a treatment plan, and intervene, if necessary, for public safety.

SUPERVISION STRUCTURE

Individual Supervision (60 minutes)

Each clinician/trainee and her/his supervisor meet for a minimum of one hour per week for scheduled, face-to-face individual supervision, in addition to the ongoing daily support and 100% availability via telephone, text message, and email. If the primary supervisor is unavailable, the delegated clinical supervisor will be available to cover the supervision. Topics reviewed in supervision can include current or potential therapy or testing case(s), and/or other assessment or treatment-related issues. Each trainee will also have access to the Clinical Director at their site 100% of the time should there be an emergency and their supervisor cannot be reached, or should they need to escalate an issue with their supervisor.

Group Supervision (two hours)

All trainees meet together weekly with a supervisor for two hours of group supervision. Group supervision provides an opportunity to strengthen professional development as well as promote peer relationship building and information sharing. If the regularly scheduled group supervisor is unavailable, a secondary group supervisor will be available to cover the scheduled group supervision.

Clinical Case Consultation (90 minutes)

Clinicians/trainees attend weekly clinical case consultations where they have the opportunity to give and receive feedback from their treatment team on client cases.

Didactic Training

Trainees are expected to attend 90-minute weekly didactic trainings. These trainings are crucial as they focus on sex offender treatment and the training and certification of SARATSO

recommended measures (LS/CMI and STABLE-2007). Trainees are encouraged to join the Association for the Treatment and Prevention of Sexual Abuse (ATSA; https://www.atsa.com/) and the California Coalition on Sexual Offending (CCOSO; https://ccoso.org/) to further promote ongoing professional development.

CLINICAL SUPERVISOR RESPONSIBILITIES

A Clinical supervisor is an integral part of the functioning of the HOPE Program. The supervisor provides supervision in accordance to following the guidelines of the Board of Behavioral Sciences (BBS; https://www.bbs.ca.gov/), Board of Psychology (BOP; https://www.psychology.ca.gov/), and universities supervision requirements for each of their assigned supervisees. The clinical supervisors are responsible for upholding the Code of Ethics for sex offender therapists, governed by psychology associations and boards such as CASOMB (https://casomb.org/index.cfm?pid=1215) and ATSA (https://www.atsa.com/).

The primary clinical supervisor is required to be a licensed mental health professional (LMHP). Secondary consultation can be provided by unlicensed, but license eligible clinicians, who hold CASOMB certification. It is the responsibility of the primary clinical supervisor to be up-to-date on the caseloads of each of their supervisees. This includes knowing the names of each client and risk profile. A supervisor is to understand if the client is on a behavioral contract or a financial agreement, the client's polygraph status including date of next polygraph, the client's overall level of participation in treatment including attendance, and anything else that might be considered clinically significant.

Clinical supervisors ensure that their supervisees are following the policies and procedures of this manual and compliance with our contracts, including, but not limited to, meeting our due dates and documentation requirements. Supervisors review, provide feedback, and approve all clinical artifacts, including, but not limited to the following:

- Daily attendance, Missed Status Alert (MSA) and Status Alert (SA) documentation
- Reviewing and finalizing session notes
- Monthly Treatment Reports (MTRs)
- Quarterly Treatment Plans (QTPs)
- Assessments

- Progress Reports
- Treatment Modifications
- Ensuring collateral notes are completed for clients on a monthly basis

In addition to providing supervision, the supervisor must be available to their supervisee 100% of the time by phone, email, etc. Listed below are additional supervisory duties and responsibilities. Other items can be added at the discretion of the Executive Clinical Director.

Additional Supervisor Responsibilities Include:

1. Supervisor will provide supervision according to the rules and regulations of the Board of Behavioral Sciences and/or Board of Psychology;
2. Supervisor will provide individual and/or group supervision;
3. If the Supervisor must miss a scheduled supervision session, either individual or group, it is the Supervisor's responsibility to initiate and schedule a 'make up' supervision session or reassign supervision for the week to another supervisor, as appropriate;
4. Supervisor will assess the Supervisee's developmental and skill level and provide constructive feedback to support the Supervisee's professional growth;
5. Supervisor will provide the Supervisee with guidance on both the content and process of his/her clinical work. This will include concrete directives and suggestions for therapeutic strategies, theoretical frameworks for conceptualizing the clinical work, and analysis of countertransference and issues of human diversity;
6. Supervisor will raise issues of job performance and prepare a plan of improvement if deemed necessary. Supervisor will accordingly recommend additional personal support as deemed necessary to enhance clinical work performance;
7. Board of Behavioral Sciences and/or Board of Psychology paperwork submitted by the Supervisee will be completed prior to the Supervisee's departure from HOPE Program;
8. Supervisor will document a summary of content of individual and/or group supervision and track the Supervisee's attendance; and
9. Supervisor will follow all policies and procedures related to risk management and expeditiously address all risks of liability to the program or agency. Supervisor will inform the Supervisee of the extent of what are identified as liability issues.

RESPONSIBILITIES OF SUPERVISEES:

Supervision is a forum for supervisees to gain clinical, emotional, and administrative support for their development as mental health professionals. At HOPE Program, we value a team approach to both leadership and service provision. Supervisor(s) may share information to ensure quality client care and continued supervisee support.

Supervisee Responsibilities Include:

1. Supervisee will identify reasonable goals and training needs with Supervisor;
2. Supervisee will attend weekly individual and group supervision(s) as scheduled. If Supervisee must miss a supervision session, it is their responsibility to initiate and schedule a "make-up" supervision time;
3. Supervisee will plan to use and utilize the allotted individual and group supervision sessions by coming prepared with a clinical agenda. Clinical Supervision will focus on the Supervisee's clinical interventions and/or countertransference;
4. Supervisee is expected to follow the directives and suggestions of the individual and group supervisors regarding clinical issues, legal obligations and ethical responsibilities;
5. Supervisee is expected to be on time to individual and group supervision;
6. Supervisee will submit the Summary of Experience logs to the Supervisor for review and signature on a monthly basis. The Summary of Experience logs and Experience Verification forms should be an accurate reflection of the Supervisee's actual services provided. All Board of Behavioral Sciences (BBS) and/or Board of Psychology (BOP) documentation will be completed prior to the Supervisee's departure from HOPE Program;
7. Supervisee will inform Supervisor of any ethical or legal issues that arise in therapy with the client and/or family of the client in accordance with HOPE Program policies and procedures, and in the next supervision.

NEXTEST SECURITY CLEARANCE

All certified provider agencies and agency employees who have access to criminal justice information must meet The Federal Bureau of Investigation (FBI) and The California Department of Justice (DOJ) requirements by taking and passing the nexTEST exam. Prior to the

exam, you will review Criminal Justice Information Services (CJIS) security protocols, such as not leaving out your laptop for others to view confidential information, or need to know versus want to know. In compliance with CASOMB policies, HOPE Program requires all staff to undergo the nexTEST security clearance test every two years to ensure compliance with the Criminal Justice Information Services (CJIS) security protocols.

Upon completion of the nexTEST exam, staff will upload their certificate to their Human Resources (HR) portal in TriNet. Staff will NOT send CASOMB a copy of their certificate; the system will automatically update once the test is complete. Click here to follow step-by-step instructions to complete your NextTEST:
https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/training/casomb/nextest-aka-cjis

TREATMENT MODALITIES

Various treatment modalities are employed when working with individuals convicted of a sexual offense. In this section, we will discuss some of the most effective models used in managing sexual offenders in the community. The most effective frameworks include the following:

- The Containment Model
- Risk Need Responsivity (RNR) Model
- Offense Process Models

CONTAINMENT MODEL

The Containment Model is an interdisciplinary strategy to manage sexual offenders that combines elements of treatment, supervision, monitoring, and risk management. It includes interventions designed to produce behavioral changes that are supported by modifications in the offender's daily lifestyle and psychological condition. The Containment Model is built upon two observations:

1) The majority of criminally detected sexual offenders eventually return to communities under some form of mandated community supervision

2) The likelihood of re-offense is reduced when well-planned mechanisms are in place that support full and productive participation of offenders in community life, while holding them accountable for the harm they have created and for the choices they made and continue to make.

The model is implemented on a case-by-case basis through collaboration of the various stakeholders involved in each case. For example, a team that includes a parole agent, a specialized law enforcement officer, inpatient and outpatient treatment providers, a medical doctor, a polygraphist, a conditional release supervisor, and in some cases, a victim advocate acts to "contain" the rehabilitating sexual offender. In some cases, a responsible family member or friend may be integrated into this model. Each agent thoroughly learns the sexual offender's pattern of sexual offending, participates in ongoing regular discussions that identify any resurfacing of key risk factors, and collaboratively strategizes and coordinates services (e.g., level of treatment, probation, etc.) to interrupt the cycle of re-offense.

> *The Containment Model asserts that reduction in recidivism is achieved only when the multiple criminal justice agencies involved in each case proactively strategize to close the gaps that naturally occur when multiple systems work together to promote change. A thorough understanding of, and full participation in the Containment Model by all involved agencies is essential to public safety and vital to the success of externally motivated change. Ideally, externally motivated change can be internalized and the offender will continue to live a prosocial lifestyle when supervision ends and legally mandated monitoring mechanisms are discontinued (p. 4). (CCOSO Adult Standards Guidelines, 2010).*

HOPE Program operates under the strict adherence to the Containment Model, as a collaborative approach to sex offender management. Per California Penal Code 290.09, "The certified sex offender management professional shall communicate with the offender's probation officer or parole agent on a regular basis, but at least once a month, about the offender's progress in the program and dynamic risk assessment issues, and shall share pertinent information with the certified polygraph examiner as required." The Containment Model must use a victim-centered approach. The three primary stakeholders who have regular communication are:

1. Participation and open communication by a **supervising officer/agent**
2. Participation and open communication by **HOPE Program**
3. Participation and open communication by the **polygraph examiner**

These three stakeholders are the core of the Containment Team, although other team members could participate in the Containment Team Meeting (CTM) to share general information, as mentioned above (e.g., the registering law enforcement agency or victim advocates).

Prior to entering treatment, all clients are required to sign a Release of Information (ROI) consenting to open communication (releasing and exchanging) of all aspects of their treatment to and from HOPE Program and their supervising officer and/or the courts. HOPE Program is responsible for ensuring that the client undergoes a comprehensive evaluation that includes standardized risk assessments, such as the STABLE-2007/ACUTE-2007 and the LS/CMI. Initially, and throughout the client's tenure in treatment, their risk scores, cooperation with treatment and program policies, including progress in the program and/or any concerns regarding the client's risk, will be communicated to the supervising officer.

Client's treatment contract describes the responsibility of the client to avoid risky, aggressive, or re-offending behavior and high-risk situations. The treatment contract requires the client to self-report any such behaviors to HOPE Program and their supervising officer as soon as possible. Below is an overview of the Containment Model and how HOPE Program operates within the containment team.

Containment Team



KEY PREMISES

- Sex offenders eventually return to their communities under some form of supervision.
- Reduced likelihood of re-offense when offenders experience productive participation in community life and are held accountable for the harm they have created and the choices they make.
- Reduction in recidivism is achieved only when the multiple criminal justice agencies involved, proactively strategize to close the gaps that naturally occur when multiple systems work together to promote change

CONTAINMENT MODEL STAKEHOLDERS

1. Criminal Justice System
2. Treatment Provider
3. Polygraph Examiner
4. Victim Advocacy

1. CRIMINAL JUSTICE SYSTEM

KEY STAKEHOLDERS

- Law enforcement
- Supervising officers/agents
- Parole
- Probation
- Courts

Role: Supervision

- Primary Case Manager - oversee and coordinate the overall management of sex offenders in the community
- Ensure ongoing adherence to the conditions of parole or probation
- Apply pressure through clear expectations and through the use of sanctions to ensure the offender complies with their supervision requirements
- Issue authorization to take parole or probation violators into custody
- Enforcer of the court
- Perform routine house visits
- Maintain regular contact with friends and family
- Monitor the sex offenders' activities
- Restricts activities/rewards progress of offender
- Thoroughly learn the sexual offender's pattern of sexual offending
- Participates in ongoing discussions with the treatment team and polygraphist
- Report any risk factors and concerns
- Uses victim impact information
- GPS - may help monitor the location of offenders and provide information during the investigation of new sexual offenses

2. TREATMENT PROVIDER

Key Stakeholders

- Evaluator
- Individual Psychotherapist
- Group Facilitator

- Psychiatrist (if needed)

Role: Evaluate, Reduce Re-offense, & Support

- Thoroughly assess offender and identify risk factors
- Thoroughly learn the sexual offender's pattern of sexual offending
- Help offenders develop internal control, and understand and interrupt their individual offense cycles
- Identify cognitive distortions / offense supportive schemas
- Maintain close collaboration with the supervising officer and polygraphist by having ongoing regular discussions
- Report any risk factors

3. POLYGRAPH EXAMINER

Polygraph Testing – a treatment tool used to increase disclosures, ensure adherence to treatment goals, and to increase compliance with conditions of probation or parole.

Key Stakeholder

- Certified Polygraphist

Role: Monitoring

- Used to enhance the assessment process
- Helps break down offender denial
- Helps monitor the offender's deviant fantasies and external behaviors, including access to victims
- Investigates compliance with probation or parole, and the treatment program's policies
- Report findings to stakeholders

4. VICTIM ADVOCATE

Key Stakeholders

- The Child Abuse Listening, Interviewing and Coordination Center (CALICO)
- Victims of Sexual Abuse
- Spouse or Partner
- Family
- Circles of Support & Accountability (COSA)

Role: Victim-Centered & Victim Impact

- Provides support, services, and information to victims
- Helps offenders understand the victim's perspective
- Act as a liaison between the victim and the containment team
- Provides training and input into public policy
- Express concerns about a sex offender's re-entry into the community

SUMMARY

Individualized and implemented on a case-by-case basis through collaboration of the various stakeholders involved. Interventions are designed to:

- Produce behavioral changes
- Modify offender's daily lifestyle
- Reduce risk of re-offending
- Create healthier, pro-social outlets

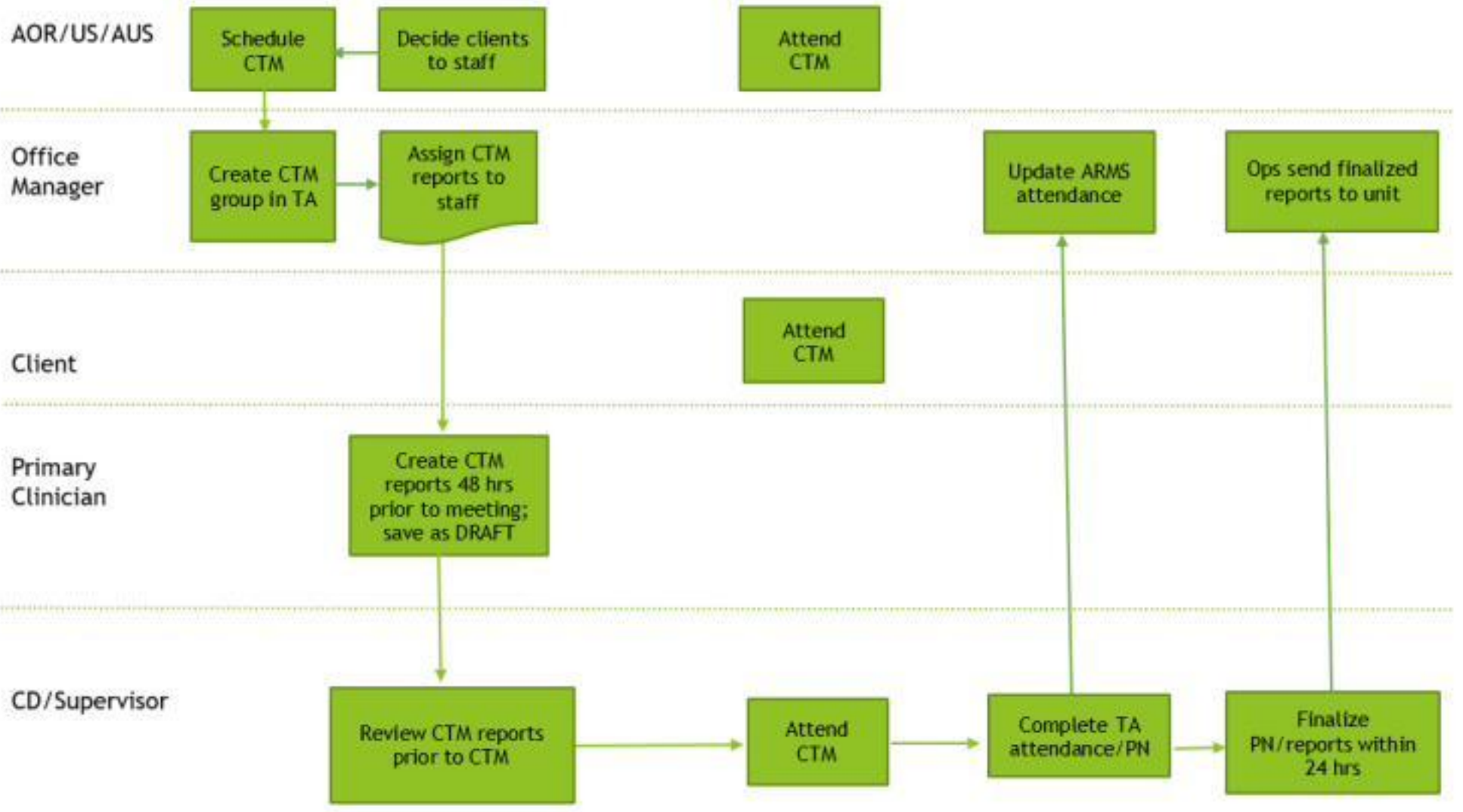


COLLABORATION WITH SUPERVISING OFFICERS USING THE CONTAINMENT MODEL APPROACH

While HOPE works collaboratively with all stakeholders, the agency works most closely with the Polygraph Examiners and Criminal Justice System. In regards to polygraph examiners, HOPE works closely with polygraphists to support the use of specialized assessments that enhance treatment and determine compliance with release conditions. In conjunction with the Criminal Justice System, it is imperative that HOPE maintains constant communication with supervising officers regarding any information that we believe might be material to the case, the defendant/offender, or to the safety of any person or group. In return, supervising officers will notify HOPE of ongoing investigations, or other areas of concern that are relevant to our mutual client. Any information disclosed at any time by the supervising officer or agent regarding an active investigation, pending charges, surveillance of a client, or any information that is deemed confidential or sensitive in nature cannot be disclosed to the client in therapy under any circumstance.

HOPE also facilitates weekly or monthly Containment Team Meetings (CTMs) either at HOPE or at the supervising officer's location (i.e., probation or parole office). Referring officers, polygraphists, and HOPE treatment team are invited to participate. Cases are presented and discussed amongst the group. Similar to above, it is important to note that if information discussed during a CTM pertains to a client being under an active investigation or if any other matter that is deemed sensitive is discussed or obtained by the supervising officer or agent, then under no circumstance can that information be disclosed to a client. The diagram below provides an overview of the steps involved in scheduling and conducting a weekly or monthly CTM.

CONTAINMENT MEETING PROCESS FLOW



HOPE Program believes that effective communication between the primary mental health provider and the referring supervising officer is a key success factor in implementing the Containment Model on which our treatment philosophy is based. HOPE will make every reasonable effort to stay in regular contact with the referring officer, so as a matter of general practice, formal notifications called "Status Alerts" and "Missed Session Alerts (MSAs)" are

generated in our electronic client file system known as TreatAnyone (TA). The chart below demonstrates the minimal expectations between the supervising officers/agents and HOPE representatives during a CTM, followed by HOPE's CTM agency representative guide.

GENERAL OVERVIEW & EXPECTATIONS OF CONTAINMENT TEAM MEETINGS (CTMS)			
SOT (Sex Offender Treatment) Provider	**Supervising Officer(s)**	**Polygrapher**	**Victim Advocate**
Arrive to CTM on time	Arrive to CTM on time	SOT provider to review polygraphist report to discuss exam findings with stakeholder	The victim advocate can sit in on CTMS, if applicable
Present attendance history	Discuss supervision compliance	Discussion of exams can help SOT provider and stakeholders monitor the offender's deviant fantasies and external behaviors, including access to victims	Advocates can provide support and information to the victims
Discuss participation in treatment	Relay any violations of conditions	Discussion of exams helps to investigates compliance with probation/parole	Advocates can help offenders understand the victim's perspective
Discuss polygraph results	Discuss clients' source of income	SOT provider to make recommendations for future polygraph type	Victim advocates act as liaison between victim(s) and containment team
Discuss any changes in LS/CMI and STABLE risk factors (i.e., increase, decrease, or no change)	Relay whether client is employed	SOT provider to schedule polygraph examinations with client and polygraphist	Advocates can express concerns about a sex offender's re-entry into the community
Relay any behavioral concerns	Discuss clients' housing situation (i.e., stable, unstable, or transient)		
Discuss completion of treatment goals	Discuss clients' relationships (i.e., in an intimate relationship, evidence of prosocial or antisocial peers)		
Discuss completion of curriculum	Officers/AOR's to provide polygraph referral forms to relay specific questions wanted for upcoming polygraph examinations		
Discuss eligibility for titration or suggested increases in services	Discuss placement or amendment of conditions		

CTM Agency Representatives: A How-To Guide:

Attendance for Containment Team Meeting (CTM)

When marking attendance for CTM, you will use the bolded "blurbs" listed below. The only time you would delete (do not "skip") a Client (CLT) from the CTM collection and NOT mark their attendance is if the Agent of Record (AOR) cancels his/her CTM and requests that the client be rescheduled. If this is the case, save the Progress Report and Treatment Plan update (i.e., CTM report) as "work in progress" to use at a later date. You would inform the clinician the client's meeting was postponed, and ask them to edit the report to include updated information once that client is rescheduled. As a courtesy, you can inform your operation's team as well so they are aware as to why you did not finalize the report. Finally, if anyone not on the CTM list was discussed for any reason (e.g., to gain collateral, PO/AOR provides custody updates), then you will NO longer add them to the CTM group collection, but instead you will simply add the discussion to a collateral note.

- If the client attended CTM, mark them as **present** in TreatAnyone (TA) and use the dropdown menu to add the template seen below.
    - **Client (CLT's full name should populate here; John Smith) is present and staffed. Refer to the containment report dated 3-15-2022.**
- If the client did not attend, mark as **excused (NOT no-show as you would in traditional progress notes)** in TA and use the dropdown menu to add the template seen below.
    - **Client (CLT's full name should populate here; John Smith) was staffed. Refer to the containment report dated 3-21-2022.**

Progress Report & Treatment Plan Updates

Please reference the templates above or in TA for awareness of the clinical expectations for all sections of the Progress Report & Treatment Plan Update report (i.e., Risk Summary, Overall Clinical Comments, Requested Frequency of Treatment, and then the final section that you complete, which is the Agency Representative Comments). Before finalizing all reports, CTM representatives are to make sure they are correcting any clinician errors in their reports. Also, if AOR provides an updated client status (e.g., incarcerated, medical, etc.) or provides an updated Agent of Record (AOR) or updated Controlling Discharge Date (CDD)/Treatment End Date (TED), please make sure you cross-reference info in the system, add a collateral note, and email

Operations@almaxconsulting.com if changes need to be made in TA. The CTM representative can edit phone numbers and CDD, for example, but any status update (i.e., absconded, incarcerated, supervision ended, receiving services, medical) needs to be edited by operations so they can update ARMS (i.e., CDCR database).

Agency Representative Comments:

EXAMPLE #1

Mr. Smith did not attend his scheduled Containment Team Meeting (CTM). His assigned Agent of Record (AOR) and the Unit Supervisor (US) of Delta GPS were also in attendance. His Static-99R is a 1 and his CSRA is a 1. He resides in a Sober Living Environment (SLE). He has a history of Methamphetamine use. He does not have a telephone and he does not engage in HOPE services; however, he is receiving Mental Health (MH) treatment through the county and works closely with a MH team. The subject also attends the Behavioral Health Reentry (BHR) program, Community Outreach, and has a conservator. The subject will remain on medical leave until more stabilized and amenable to Sex Offender Treatment (SOT). Remain SA level of supervision.

EXAMPLE #2

Mr. Doe attended his scheduled Containment Team Meeting (CTM). His assigned Agent of Record (AOR) and the Unit Supervisor (US) of Seaport GPS were also in attendance. He resides with his girlfriend in a studio apartment on California Street in downtown San Diego, CA. He is employed full-time at Amazon. He has had violations in prior review periods for failure to attend Sex Offender Treatment (SOT); however, he has not had any violations in this review period. He needs to be scheduled for his maintenance polygraph examination. AOR sent polygraph questions to operations. Maintain current supervision level.

EXAMPLE #3 (INCORRECT Sample)

The client attended his scheduled CTM **OR** the Client was present. **(This example is NOT acceptable per the Sex Offender Unit (SOU) as more detailed information is needed as seen in Examples #1 and #2 above).**

What to listen/look for and document during the meeting:

- Employed? Housing? In a relationship? Do we have the correct Static-99R score? Do they attend other groups outside of HOPE for Mental Health (MH) services through Parole Outpatient Clinic (POC), Behavioral Health Reentry (BHR) program, Telecare (We need a Release of Information (ROI) to talk to Telecare) or Veteran's Affairs (VA; we need an ROI for them as well). Do they attend Alcoholic Anonymous (AA), Narcotics Anonymous (NA), and or Domestic Violence (DV) courses? Do they attend school? Do they engage in prosocial activities (i.e., Agent of Record (AOR) might say the subject goes to the gym in his spare time or the subject reports reading and watching movies with his partner in his/her spare time)? Have they recently violated their conditions; any changes to their conditions of parole? Any updates to their status (i.e., incarcerated), supervising agent/officer, and or any updates to their discharge date. If parole gives you an updated Controlling Discharge Date (CDD) or Max CDD, you will collateral this CDD in clt file and check to ensure CDD matches his Treatment End Date (TED) in TreatAnyone (TA) under Enrollment→Enrollment Information. If the dates do not match, email Operations@almaxconsulting.com to request that Op's updates CDD in our system or edit the CDD yourself in TA labeled "Tx end date." If it is an upcoming CDD, cc operations and the assigned clinician for awareness so the clinician can prioritize assessments, begin/continue treatment planning before the client discharges, and prepare their discharge summary report.

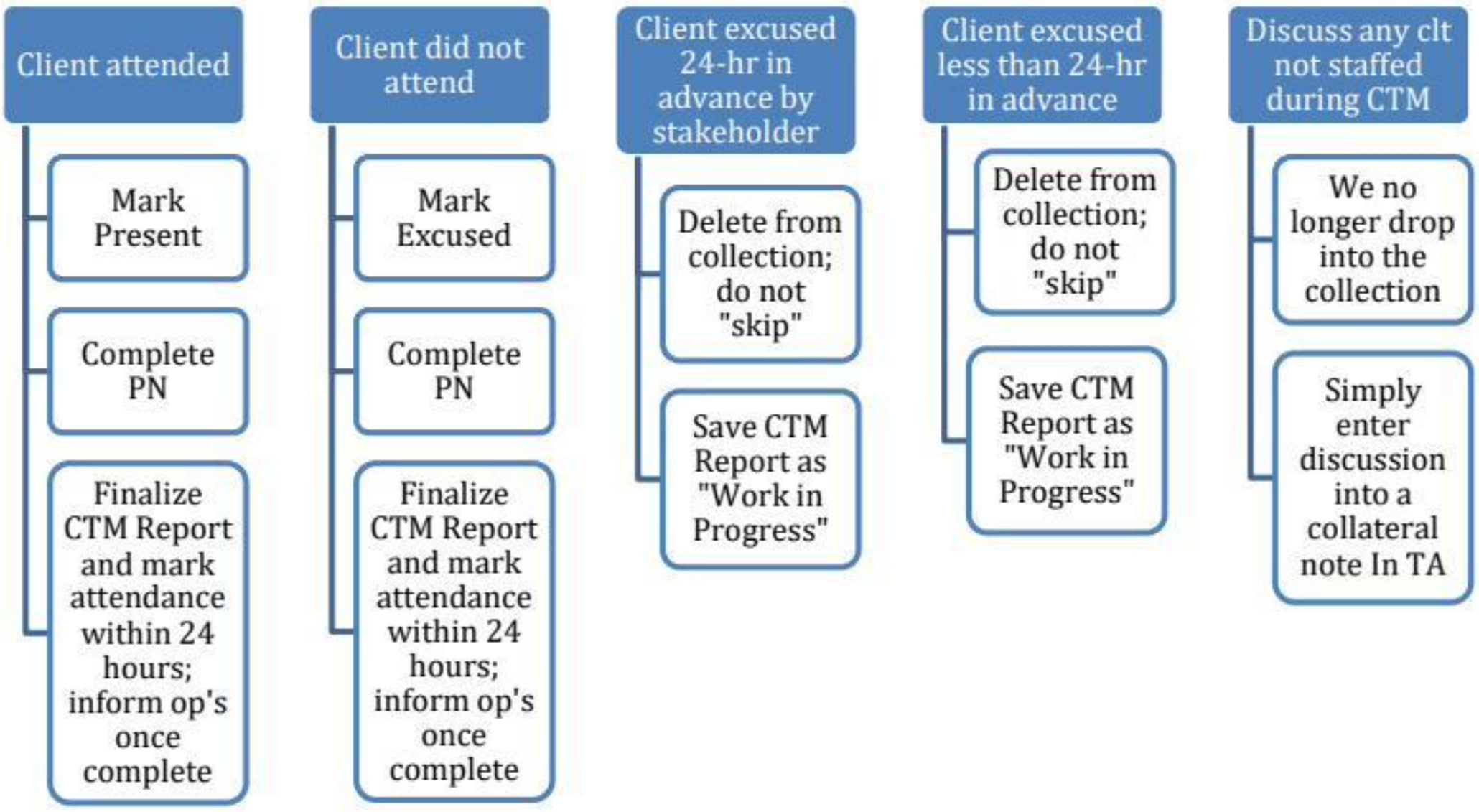
What you verbally report during CTM:

- Read clinician's reports and attempt to review ahead of time to ensure completion and accuracy. Worse case, if the report is incomplete, provide risk scores, a brief description of the client's progress and participation (review a recent group and individual note), relay polygraph results to include pre and or post-test disclosures, any risk factors or areas of noncompliance you see. Are they ready to titrate and reduce in treatment? What part of the curriculum are they in (i.e., prep, core block I or II, maintenance, or aftercare and which module) per their last group note entry?

CTM TO-DO Reminders:

- Write out words at least once then you can use acronyms (i.e. Sex Offender Treatment; SOT). For a list of commonly used acronyms, click here: https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/list-of-acronyms-commonly-used-by-stakeholdershope
- Review and edit grammatical errors prior to finalizing reports
- Follow up with Ops for any TA updates (cc clinician if necessary)
- Follow up with clinicians if parole confirms an upcoming CDD so as to remind the clinician to prioritize the client's risk assessments/Self Reports (SR's), and to engage in discharge planning.
- Collateral important things to know/track (i.e., confirm CDD's, Max CDD (max means the CDD cannot be extended even if the client gets a violation).
- Follow up with clinician's missing reports. If it becomes a theme, loop in supervisors for additional reinforcement.
- Identify themes and bring to staff meeting if many are not doing something similar, such as including the disclosures when reporting polygraph results. Simply saying "the client passed his last polygraph" is not acceptable. The clinician needs to include whether there were disclosures or not. Appropriate completion of this section might say the following:
  - Mr. Smith's maintenance polygraph conducted on 3/28/22 resulted in no significant reactions with the following pretest disclosures:
    - Use of alcohol twice since his last polygraph examination
    - The subject reported that he is in a romantic relationship
    - He reported accessing social media via his cell phone (i.e., Facebook and Instagram)
    - He reported use of the dating app, "Plenty of Fish"
    - Occasional masturbation to adult X-rated material via the website "pornhub.com"

HOPE Representative CTM Attendance Cheat Sheet:



Containment Meeting (CTM) Reports

Probation CTM representatives no longer need to complete monthly CTM reports. Instead, the HOPE representative will add the agency feedback to the CTM collection attendance note. All CDCR clients staffed at CTMs require corresponding documentation in TA. Clinicians are to create a report titled, "**Progress Report and Treatment Plan Update"** in TA, in preparation for all CDCR clients' CTMs, 48 hours prior to the meeting. Templates are provided in TA to help guide clinicians to ensure vital elements are discussed during the CTM. See step-by-step instruction below:

How to Create a CDCR CTM Report: "Progress Report and Treatment Plan Update"

- Go to client file in TreatAnyone (TA)
- Go to Chart
- Click + add chart item in the right-hand corner

- Select "Create clinical report"
- Click clinical report dropdown and choose "Progress report and treatment plan updates" for CDCR clients
- Under "Clinical report date," enter the date of the CTM meeting, not the date the report was created
- Under "What period does this clinical report cover?" click “custom” and include the six months. Do not click “last six months” as this does not include the current month and only includes the six months’ prior
- Click Next
- Complete each section seen below and ensure you use the templates provided

1. **Risk Summary Section:**

EXAMPLE:

Mr. Doe’s risk assessments were reassessed on 4/11/22. His STABLE score reduced by 2 points in the following domains: Hostility towards women and impulsivity. His LS/CMI increased 2 points in the following domains: Criminal history and procriminal attitude.

His current **STABLE-2007** is an **8 (Moderate)** and his overall general Risk/Need Total score derived from the **LS/CMI was 21 (High).** Comparable individuals with the same LS/CMI score as Mr. Doe have, approximately, a **44%** chance of re-offending within one year.

Mr. Doe’s composite assessment placed him in **level IVa** for supervision and intervention using the standardized risk framework. Individuals placed in Level IVa are considered **Above Average Risk**. Out of 100 individuals convicted of sexually motivated offenses with the same risk profile as Mr. Doe, Level IVa would be expected to recidivate sexually (including sexually motivated breaches) at a rate of **17.8%** over 5 years.

OR For Women

Mrs. Doe's overall general Risk/Need Total score derived from the **LS/CMI** is **4**, which fell in the overall **Very Low level**. Comparable individuals with the same LS/CMI score as Mrs. Doe have, approximately, a 1% chance of reoffending within one year.

Women are not assessed using the Static-99R or the STABLE-2007, so HOPE is not able to provide a composite assessment of risk/needs to produce estimates of sexual recidivism; however, the STABLE-2007 is used to develop overall treatment goals and interventions for females. Mrs. Doe scored a **2** on the **STABLE-2007** which placed her in the overall **Low** category.

2. **Goals Section:**

   Enter a minimum of three goals. One treatment target must come from the LS/CMI, another must come from the STABLE-2007, and the third is the clinician's choice.

3. **Section One: Engagement & Participation in Treatment Section:**

   Choose from the following options: Not met, Partially Met, or Met

4. **Section Two: Progress Towards Reduction in Risk (Criminogenic Needs & Protective Factors Section):**

   Choose from the following options: Not met, Partially Met, or Met

5. **Overall Clinical Comments Section:**

   EXAMPLE:

   - **Attendance:** Mr. Doe has had perfect attendance since beginning HOPE program on 4/20/20 OR He's had five absences in the last 6 months (1/2/24, 1/5/24, 1/20/24, 2/1/24, 2/16/24); he was placed on a behavioral contract on 6/1/20 to address his attendance (Do not include % here, instead include # of absences).
   - **Current Module:** Mr. Doe began treatment on 4/20/20. He continues to participate in the **preparatory phase** of treatment and just completed **module 4 of 6** on motivation and resistance.
   - **Does the client admit to his/her sexual offense(s)?** Mr. Doe takes responsibility for his sexual offense.
   - **Polygraph Type, results, and disclosures:** Mr. Doe's last maintenance polygraph on 7/1/20 indicated no significant reactions, with the following pretest disclosures:
     - Viewed adult X-rated pornography

    - Sexually active with adult girlfriend
    - Drank alcohol with his girlfriend during COVID-19 quarantine. He added that he stopped drinking as soon as he was scheduled for his polygraph examination.
- **High-risk behaviors:** Mr. Doe reported urges to drink alcohol and smoke cannabis, but reported that it is against his conditions of parole. He added that he has refrained from use since his release from custody. A relapse prevention plan was created.
- **Concern(s) in treatment:** Mr. Doe endorsed anxiety in relation to his transition back into the community, as well as anxiety in relation to ensuring he follows his conditions of parole. Coping skills were provided.
- **Participation and progress in treatment:** Mr. Doe actively engages in discussion, without being prompted, and he appears motivated for treatment. He and his clinician continue to target his treatment goals in an attempt to reduce his risk of re-offending. A current focus in treatment has been on behavioral control skills and the events surrounding his sexual offense. Next month, healthy intimacy skills will also be a focus of discussion. Finally, despite his endorsing anxiety, he appears to be coping effectively, evidenced by challenging his cognitive distortions and positively reframing his thoughts during sessions.
- **Titration:** Mr. Doe is not eligible for titration at this time. He is in the preparatory program (Module 4 of 6) and still needs to complete blocks 1 and 2 of the core curriculum before titration to the maintenance phase of treatment can be considered. Upon successful completion of HOPE's curriculum, Mr. Doe may be eligible for titration.

6. **<u>Requested Frequency of Treatment Section:</u>**

EXAMPLE #1

Remain the same frequency of treatment

EXAMPLE #2

In addition to Mr. Doe's current level of treatment (bi-weekly maintenance groups and monthly individual sessions), this writer is proposing an increase in services to include

monthly couples therapy sessions (i.e., once per month). Specifically, Mr. Doe has reported ongoing marital conflict and he has requested couples counseling, in addition to his current level of services, in hopes of gaining healthy conflict resolution skills and healthy intimacy skills with his wife. Being that domestic violence was a nexus to Mr. Doe's sexual offense, this writer is recommending additional services to better adhere to the risk principle and to further reduce his risk of re-offending.

EXAMPLE #3

Upon completion of HOPE's curriculum, this clinician will present Mr. Doe to the treatment team to propose a reduction in treatment. Specifically, this writer would like to transfer Mr. Doe to HOPE's bi-weekly maintenance phase of treatment. Mr. Doe is expected to complete his last module of Core 2 Block 2 within the next few weeks.

7. **Agency Representative Comments Section:**

   This section is left alone and completed by the representative attending CTM. Once all sections of the report are complete, the clinician is to save the report as "Work in progress." The CTM representative (i.e., clinical director) will review and finalize the report after completing the final section. Clinicians will need to review the representative's comments upon completion of report.

RISK NEED RESPONSIVITY (RNR) MODEL

In order to provide evidence-based treatment, it is required that all services (group, individual and chaperone training) and treatment plans are delivered under the auspice of the Risk Needs Responsivity (RNR) Model.

The RNR model was developed in the 1990s by Andrews, Bonta, and Hoge and is applied to a wide range of criminal populations. RNR has become one of the most influential models guiding treatment interventions in corrections (Ogloff & Davis, 2004). Hanson, Bourgon, Helmus, and Hodgson (2009) meta-analysis of 23 recidivism studies indicates that programs that utilize the Risk-Need-Responsivity Model evidenced lower sexual and general recidivism.

Empirical Research indicates that interventions that adhered to all the three principles of RNR (i.e., targeted low risk offenders, and their non-criminogenic needs using non-behavioral

therapeutic approaches) evidenced the largest reductions in recidivism. Treatment programs that did not adhere to any of the three principles, were actually associated with a small increase in recidivism. Treatment programs that adhered to at least one of the principles showed a small decrease in recidivism. Treatment programs adhering to two principles showed larger effects. Treatment interventions that adhered to risk reduction programs, informed by psychological science generally, and RNR specifically, have been shown to reduce recidivism by 30-40% (Andrews, D.A, 2012). When all three principles are in operation, particularly in community settings, reductions in recidivism of up to 50% have been reported (Andrews & Bonta, 2006).

Treatment and supervision are most effective when tailored to the specific RNR of the client. Risk Need Responsivity has three guiding principles:

- Risk Principle
- Need Principle
- Responsivity Principle

THE RISK PRINCIPLE

- Risk – an offender's level of risk should be matched to the appropriate level of treatment intensity and supervision. There are two aspects of the Risk Principle –
- First – Criminal behavior can be predicted
- Second – Match the level of services to the risk level of the offender

  Risk should be determined using actuarial risk assessments:

  1. Static – unchangeable factors
  2. Dynamic – changeable factors
- Provide intensive services to higher risk clients and minimal services to lower risk clients

THE NEED PRINCIPLE

- Needs - are known crime producing factors that are targeted for intervention, because they can be changed through treatment
- Identifying the specific criminogenic needs or dynamic factors are the most effective interventions
- Criminogenic needs are dynamic risk factors that are proven through research to affect recidivism

Criminogenic / Non-Criminogenic Needs

Criminogenic Needs, Major Risk	Non-Criminogenic Needs, Minor Risk
Antisocial personality pattern	Low self-esteem
Procriminal attitudes	Loneliness
Antisocial relationships; supportive of crime	Depression
Family/marital problems	Major mental health disorder
Relationship problems	Physical health concerns
Lack of performance or involvement in work or school	Poor victim empathy
Lack of prosocial recreational activities	Lack of motivation for treatment at intake
Substance abuse	
Sexual preoccupation	
Atypical sexual interests	
Emotional congruence with children	
General self-regulation	
Resistance to rules/supervision	
Poor problem solving	

THE RESPONSIVITY PRINCIPLE

- Responsivity – is matching the style, modes, and influence strategies of service with the learning styles, motivation, aptitude, and ability of cases (Andrews, 2000).
- Deliver information in a way that the client will receive the information.
    - Treatment and interventions should be delivered in a manner that is consistent with the offenders' learning styles, abilities, and personal circumstances.
    - This includes a person's learning style, motivation, and level of functioning.
- Cognitive-behavior and cognitive social learning methodologies are reported to be the most suitable (Andrews & Bonta, 2003).

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OFFENSE PROCESS MODELS

In concert with the Risk Need Responsivity Model, Offense Process Models provide CBT interventions that are used to tailor treatment to specific needs of clients. These models include:

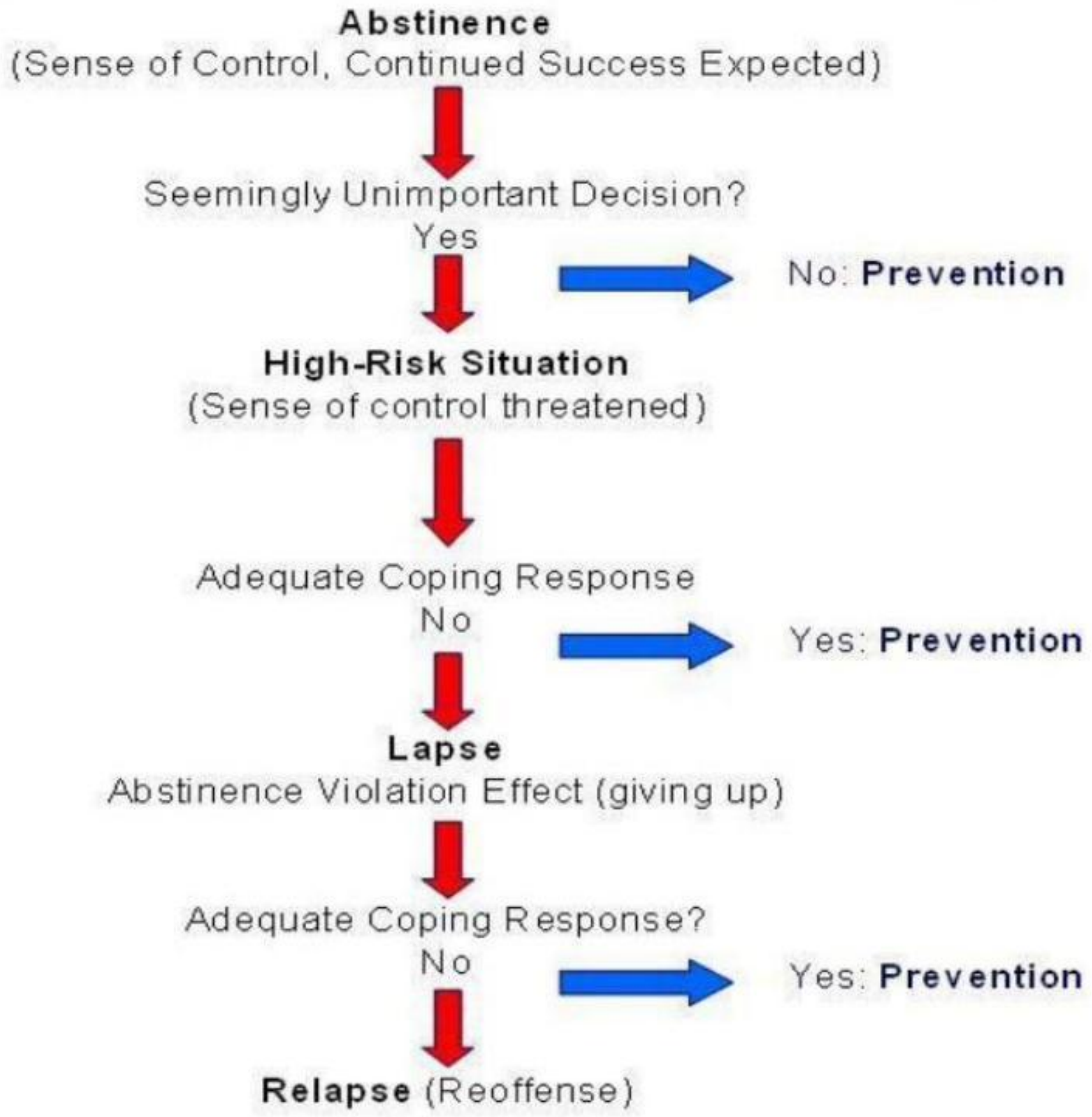
- Relapse Prevention (RP)
- The Good Lives Model (GLM)
- Self-Regulation Model (SRG)

RELAPSE PREVENTION THERAPY (RP)

RP is a cognitive behavioral approach that was originally designed for use in conjunction with treatment of addictive behaviors. More recently, it has been used as a stand-alone technique designed to teach patients how to alter their behavior to recognize and cope with triggering events, situations, and feelings that are likely to lead to a relapse.

The conceptual framework of the relapse process is useful in treating sexual offenders, who commonly experience high-risk situations by being unaware of how a seemingly benign decision can threaten their perceived sense of control and self-efficacy. Figure 1 below illustrates the Offense Chain, a model we use to help our clients develop effective coping responses and avoid inadvertently progressing along the risk continuum toward a re-offense.

Figure 1: The Offense Chain

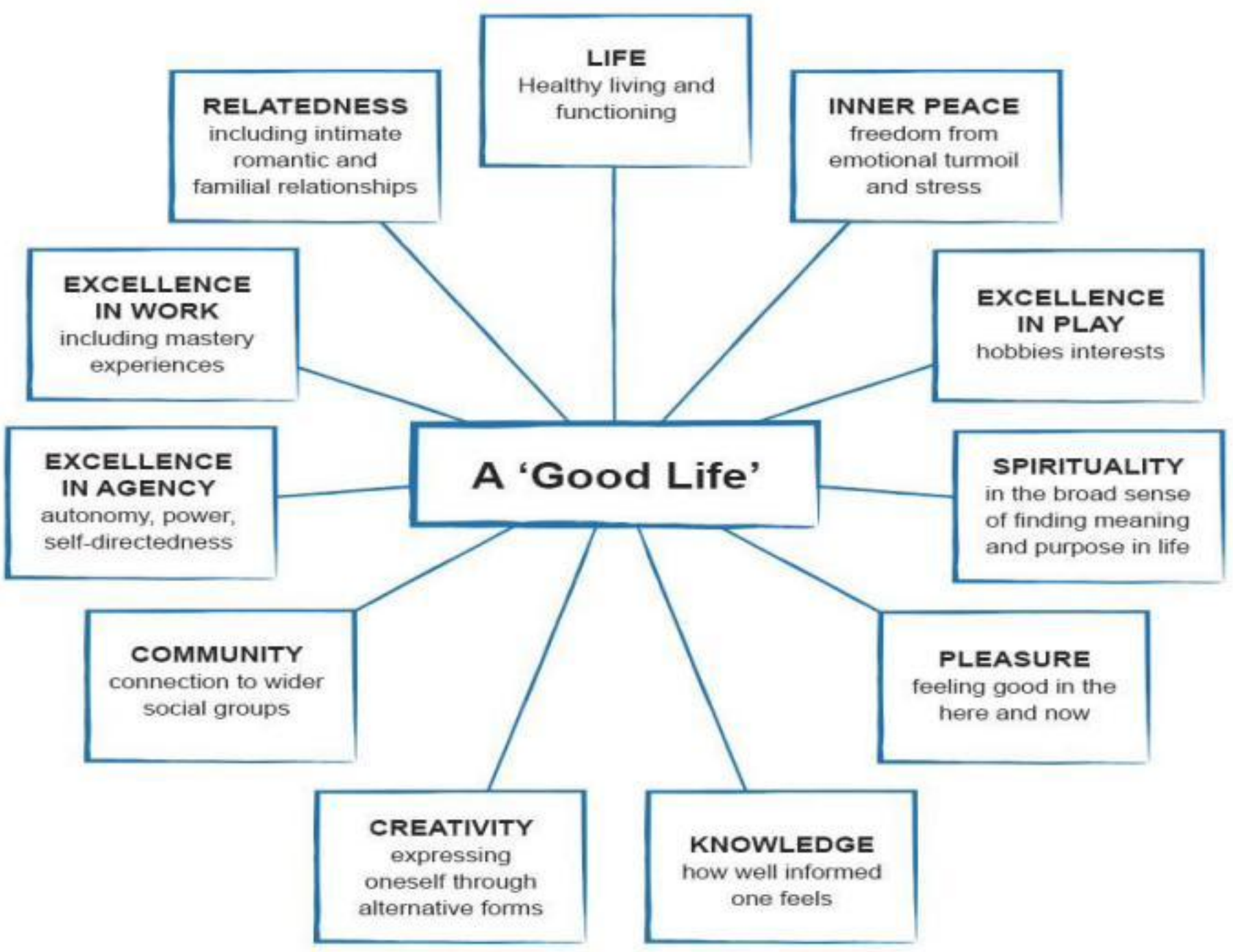


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THE GOOD LIVES MODEL

GLM is a strength-based treatment construct based on the notion that sex offenders are both holders and violators of human rights. It suggests that humans require certain "goods" in principal areas of life in order to feel whole: Life/Healthy Living, Knowledge, Excellence in Work and Play, Excellence in Agency, (self-determination and self-agency), Inner Peace, Relatedness (family/friends), Community, Spirituality, Happiness, and Creativity. The HOPE treatment program leverages this model to help clinicians understand and evaluate the risks factors associated with clients' inability to achieve the basic Human Goods. Additionally, we strongly favor this treatment philosophy because it also addresses clinician attitudes toward the offender and the relationship between these attitudes and factors, such as forgiveness, evil, and therapeutic alliance. Figure 2 below illustrates A 'Good Life' Diagram.

Figure 2: A Good Life Diagram



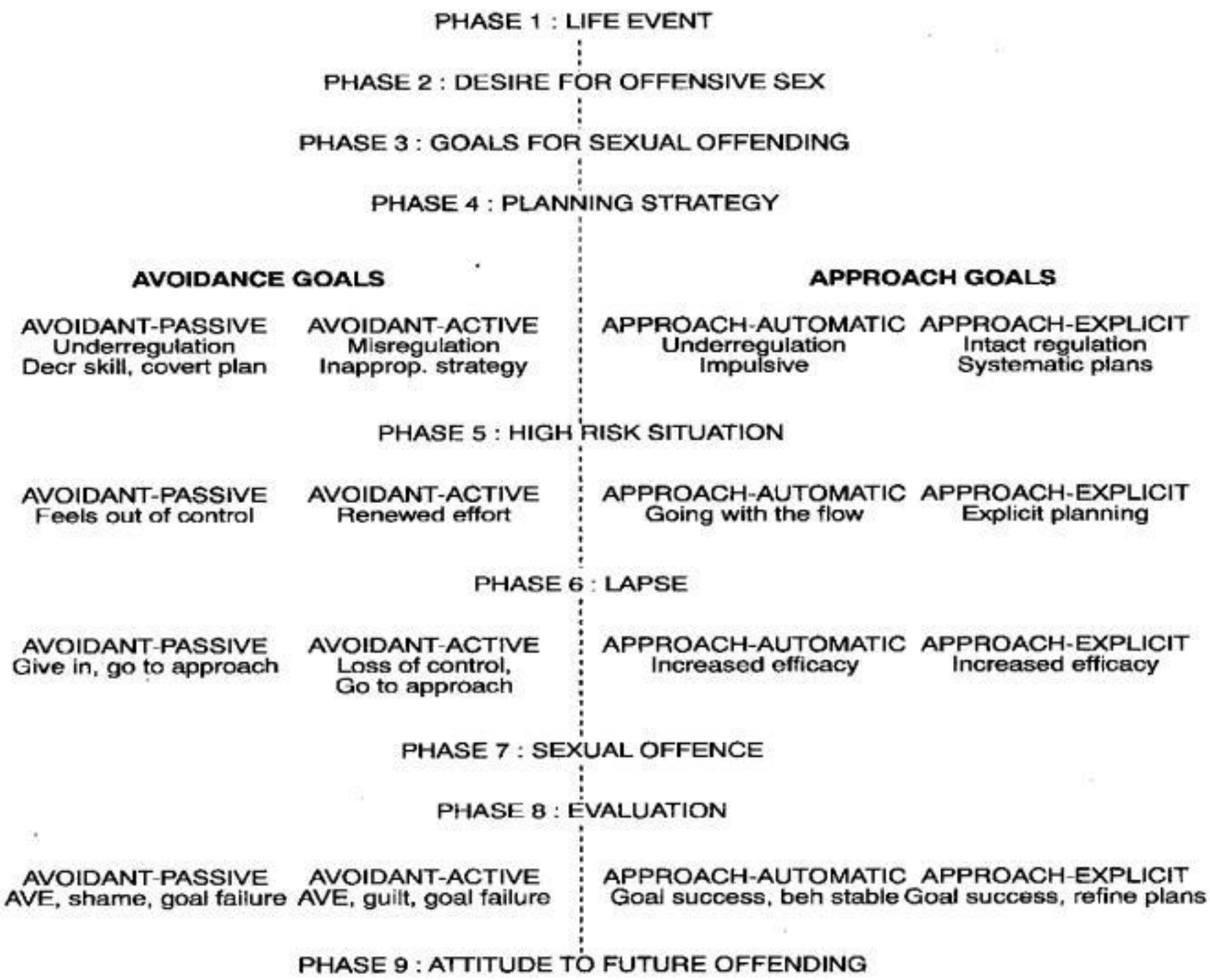
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SELF-REGULATION MODEL

According to Yates and Ward, "We argue that none of these models singly is sufficient to guide the assessment and treatment of sexual offenders, and that an integrated model that draws on research and practice in the development of case formulation, is most likely to be effective in achieving the goals of reduced recidivism, risk reduction, and reduced rates of sexual victimization (Ward and Yates, 2008)."

The Self-Regulation Model (SRM) described by Ward, Hudson, and Keenan (1998) differs from the Relapse Prevention (RP) model by identifying four offending pathways rather than a single pathway, by identifying offense goals, and by identifying the strategies used to address these goals. Figure 3 below illustrates the Self-Regulation Model.

Figure 3 Self-Regulation Model



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PSYCHOTHERAPEUTIC TREATMENT APPROACHES AND METHODS

HOPE Program utilizes evidenced-based treatment approaches when working with our clients. As such, Cognitive Behavioral Treatment (CBT) is a primary modality. All clinicians are trained to engage the client in treatment by creating a therapeutic alliance using Motivational Interviewing (MI) techniques.

TREATMENT STRATEGIES FOR SUBTYPES OF OFFENDERS

HOPE Program utilizes an individualized approach and tailors each client's treatment to his/her learning style and treatment needs. They are assigned to a treatment team and services are delivered within the Responsivity Principle.

TREATMENT

HOPE Program's treatment is developed and targeted towards the following treatment needs:

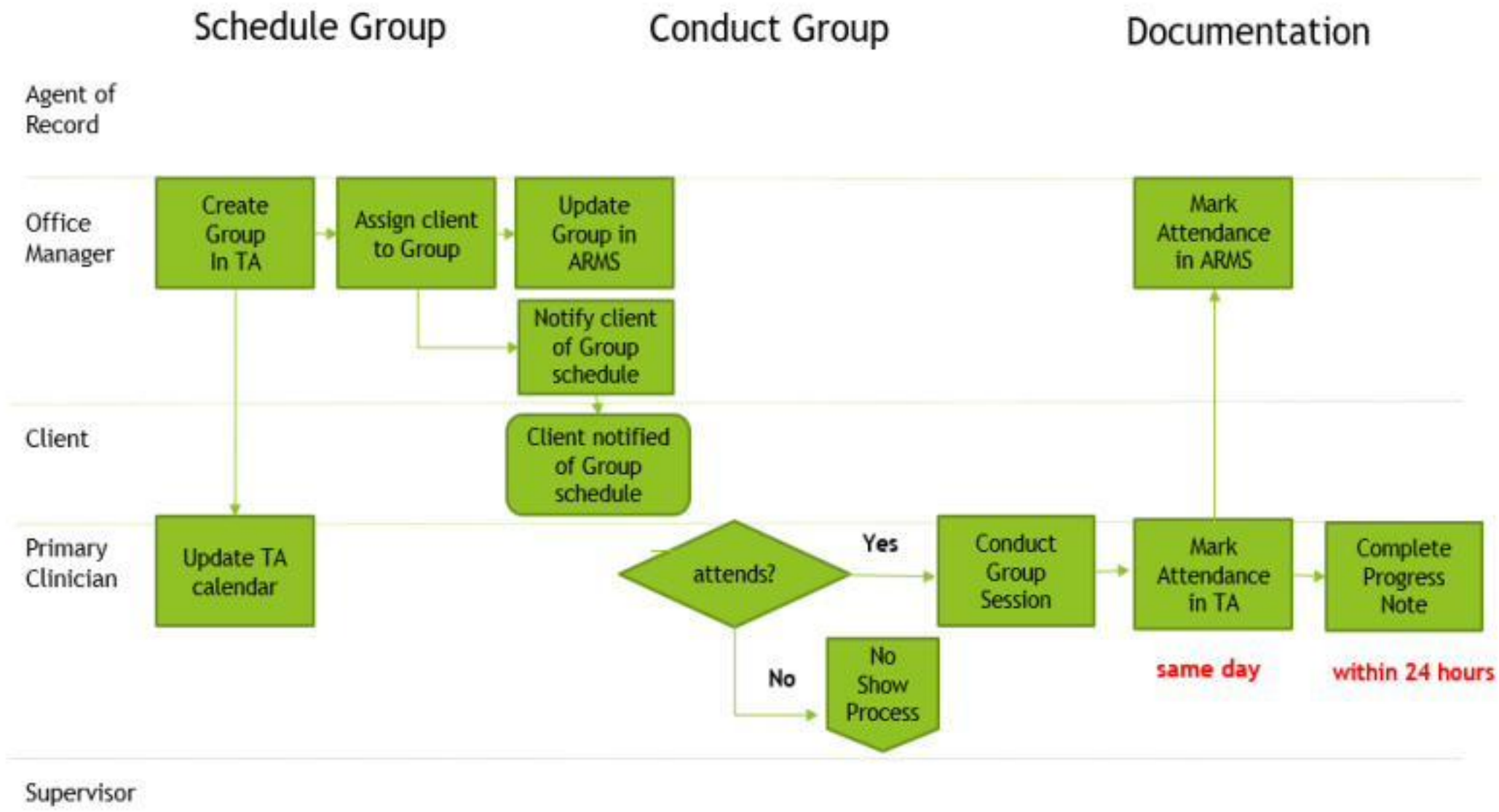
- Preparatory Treatment Program
- Core Treatment Program – Males
- Core Treatment Program – Females
- Maintenance Treatment Program
- Aftercare Treatment Program
- Adolescent Treatment Program
- Child Pornography
- Responsivity
    - Mentally ill individuals
    - Developmentally Delayed individuals
    - Illiterate individuals
    - Spanish Speaking individuals

GROUP TREATMENT

Group Treatment is provided on a weekly, twice weekly, or bi-weekly basis (i.e. every other week), depending upon the risk and need of the client. HOPE Program's groups are structured to be open-ended and rolling, not closed, which means that clinicians can incorporate different

modules or phases of treatment with new members joining a group. All groups are 90 minutes in duration. See group session process flow below.

Group Session Process Flow



INDIVIDUAL AND FAMILY TREATMENT

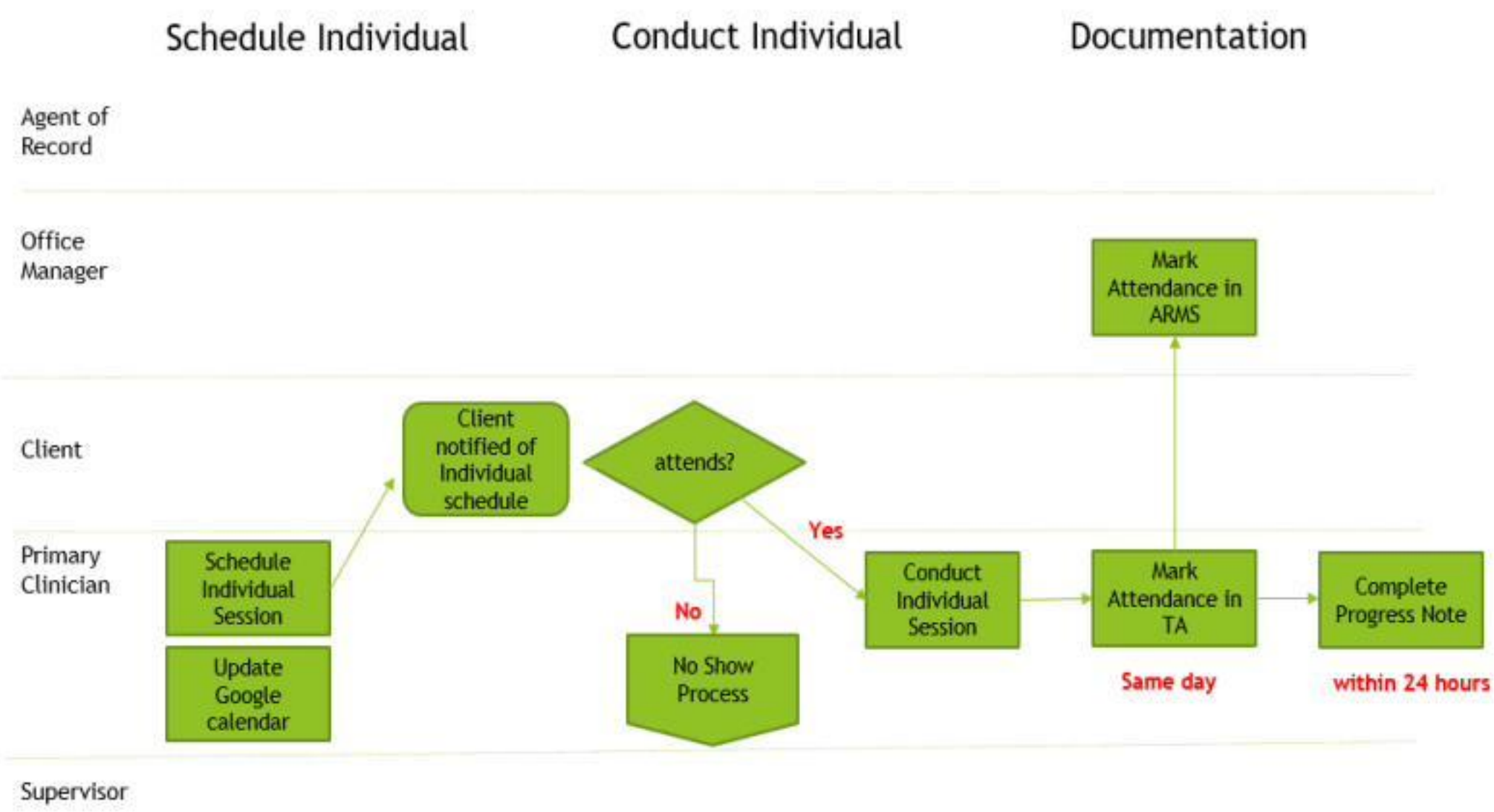
Individual treatment is offered on a weekly, bi-weekly, monthly, and quarterly basis, depending upon treatment needs. Individual treatment is 60 minutes.

Probation clients are initially enrolled in **weekly groups and weekly individual sessions** for a period of 6 months. During this time, the clinician will establish rapport, conduct a proper functional assessment/case conceptualization that incorporates risk-relevant factors and treatment need, and commence with skills-based strategies to promote risk reduction. After the 6-month period, the client is eligible for participating in **weekly groups and monthly individual sessions** if he/she exhibits appropriate behavior in group and shows progress toward treatment goals, including a reduction in identified criminogenic needs.

CDCR clients are initially enrolled in once or twice weekly groups, which is dependent upon their Static-99R score, and monthly individual sessions. Similar to probation clients, CDCR clients will participate in a functional assessment and treatment planning, but can only decrease their sessions once they have successfully completed the preparatory and core phases of treatment.

Family therapy is offered to all client types on a weekly, bi-weekly, monthly, and quarterly basis with the exception of CDCR clients. CDCR clients can only participate in family therapy once they have titrated to the maintenance phase on treatment. Once reduced to the maintenance phase, and if clinically indicated, CDCR client are eligible for once monthly family therapy sessions if approved by CDCR and HOPE's treatment team. See individual session process flow below.

Individual Session Process Flow



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MODIFICATIONS TO TREATMENT FOR INDIVIDUALS WITH LOW COGNITIVE FUNCTIONING

When clients with low cognitive functioning are enrolled into treatment, HOPE clinicians adjust their approach and cognitive behavioral therapy interventions and materials to best align with the client's cognitive level of functioning. Applying this responsivity principle where clinicians tailor interventions to a client's learning style, motivation, abilities, and strengths, maximizes a client's ability to learn.

ACCESSIBILITY FOR NON-ENGLISH-SPEAKING OR AMERICAN SIGN LANGUAGE INDIVIDUALS

If we are unable to provide services to clients in their preferred language, an interpreter is brought in either at the expense of HOPE, the stakeholder, or the client, depending on the referral source.

FOREIGN LANGUAGE INTERPRETER SERVICES

For **all CDCR clients**, language translation services MUST be coordinated through the parolee's agent of record, with the exception of American Sign Language (ASL), which is conducted through Boostlingo, a service we will discuss in more detail below. For more information on utilizing CDCR's language translation services, click here: https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/cdcr-foreign-language-interpreter-services

For **county and federal clients**, foreign language and ASL translation services will be conducted through Boostlingo.

Boostlingo is an agency that provides HOPE Program with on-demand translation services, 24/7. We can utilize this agency for ASL **(all client types, including CDCR)** and foreign language translation for county and federal clients ONLY. If we do not receive a response/confirmation about a language translator from a CDCR agent, we CANNOT use this agency just because the clinician did not want to cancel the CDCR appointment.

Boostlingo works on a rating system. After the clinician has completed their call, they will rate the interpreter and the call quality. Rating the interpreter will increase the chances of being connected with that same interpreter in the future, or not be connected if rated as having a bad experience. The call quality is in regards to the quality of the call (e.g., was there background noise, did the call constantly cut out, etc.). This agency controls all quality through ratings, so clinicians or staff in training are to provide ratings each time so they can ensure their services continue to meet expectations.

When using Boostlingo, we recommend using Chrome because it's fast and easy to use. Internet Explorer doesn't support Web RTC and will not place calls. Other browsers that will work include the following:

- Firefox
- Safari 11 (older versions won't work)
- Microsoft Edge

The steps below detail how to use the service with teletherapy clients:

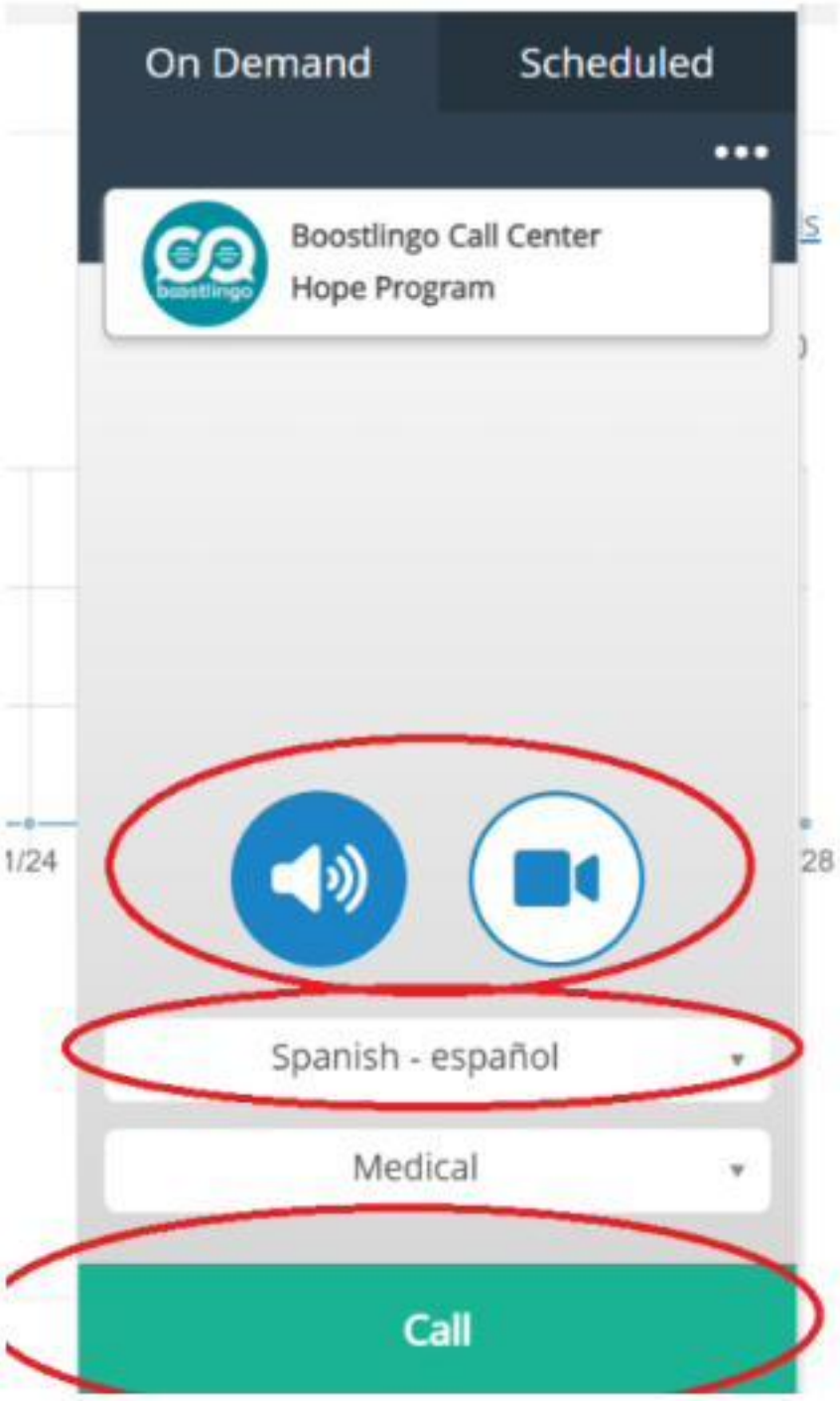
Process

1. Log into your account app.boostlingo.com using your email address and password.
2. You'll see a pop-up window on the right-hand side - select phone or video.
3. Select the language or ASL.
4. Click Call.
5. You will be connected to an interpreter.

To add a third party to a video call, initiate a video call from the Web. When your call connects to an interpreter, click the plus sign (+) in the top right of the caller. Choose from the menu.

- Dial for Audio –dials a phone number to add an audio participant. Can dial into a Zoom meeting or other third-party platform
- Share by email – If you have a default email client configured on your computer, this will generate an email invitation to join the VRI session. Enter the recipient and send. The recipient can join from any computer or phone without downloading the app. They can choose to join by Audio or video.

- Share a link to call – This option copies the URL to your clipboard. Message or email this URL to a participant so they can join from their computer or phone. They won't need any software and can choose to join by Audio or by video.



TREATMENT JUSTIFICATIONS: INCREASING/DECREASING SERVICES

In response to clients' treatment needs, his or her treatment dosage can be adjusted by increasing or decreasing their services. All clients who are referred for services have a baseline expectation of the frequency in which they participate in treatment. If a client is identified as HRSO (defined by a Static-99R score of 4 or above), the client's baseline frequency of treatment is two times (2x) per week for group treatment and one time (1x) per month for individual treatment. If a client is identified as Non-HRSO (defined by a Static score of 3 or below), the client's baseline frequency of treatment is one time (1x) per week for group treatment, and one time (1x) per month for individual treatment.

When a client demonstrates or discloses an increase in risk factors, presents with general concerns, or vocalizes a desire to receive more services to address specific issue(s), treatment services can be increased after the appropriate authorization is received from the stakeholder's unit supervisor. Clients can also decrease in services upon successful execution of their treatment goals, combined with a reduction in risk in accordance with HOPE's curriculum. Additionally, given that we are trained as general mental health clinicians first, then specialize in forensic psychotherapy, when a client presents with general mental health symptoms that may be interfering with a client's amenability to sex offender treatment, then the clinician can request increased services to address the symptoms.

To receive authorization to increase, decrease, or otherwise change the frequency of treatment, the clinician must first discuss the plan with HOPE's treatment team in group supervision or case consultation. If the team is in agreement, the primary clinician should create a treatment modification (see Treatment Modification Process training here: https://drive.google.com/file/d/1962r3wJ_fYYLTi4AStp4EyYGPGJzPAvT/view?usp=sharing) and alert their supervisor that the modification is ready to be reviewed and approved. Once the supervisor approves the treatment modification, they will send it to operations@almaxconsulting.com so HOPE's operation's team can send the modification to Parole to request approval from the Unit Supervisor (US) or Assistant Unit Supervisor (AUS) with the Agent of Record (AOR) and assigned clinician cc'ed in the email.

Please note: the new treatment dosage cannot commence until approval from Parole is received and documented in TA.

TREATMENT CURRICULUM

As a matter of best practice, HOPE Program utilizes an evidence-based curriculum to reduce recidivism and increase community safety. Sex offender-specific treatment is goal-directed and solution-focused in order to target risk factors that are associated with reoffending. All sex offender specific treatment is highly structured and guided by the curriculum. HOPE Program utilizes their own curriculum, known as Treatment Blocks, developed and written specifically for HOPE. HOPE also utilizes various supplementary materials based on each client's specific treatment needs.

HOPE Program's curriculum generally consists of four phases that gradually increase clients' skills and understanding necessary to reduce their individual risk factors and the probability of re-offending. Advancement to each phase is directly dependent on the client's ability to demonstrate measurable progress toward individual treatment goals. The measurement method is based on STABLE-2007, LS/CMI, and other specialized tools designed to assess and track change and risk over time. Clients are eligible to graduate only after they have successfully completed the Maintenance Phase of the treatment program. As noted earlier, the core curriculum integrates several evidence-based treatment modalities, such as Cognitive Behavioral Therapy (CBT), social learning theory, and motivational interviewing techniques. Moreover, RNR, GLM, as well as specific offense descriptive models are also utilized to facilitate treatment.

Progress in treatment is measured by demonstrating knowledge of the skills discussed in sessions, insight into the precipitating factors that led to their boundary violations, homework and curriculum completion, and skill application. The goal is for the client to apply the skills learned in treatment to their daily lives, thus promoting behavioral changes.

Throughout an offender's treatment, the client will be provided with different "Treatment Blocks." Each Treatment Block is specially designed to address important psychological factors, long-term vulnerabilities and risk factors, as well as healthy living skills and effective communication skills, to name a few. Each Block is composed of short learning modules, and in some cases, a module may be skipped and readdressed at a later date, pending each client's ability to address the specific topic at that time in their treatment.

To determine titration eligibility from one treatment phase to another, clinicians or staff in training will view the titration criteria (https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/hope-curriculum/titration-criteria), and if eligible, they will complete the clinical titration tracking form located under reports in TA. Once complete, the assigned clinician will present the client's case during group supervision or clinical case consultation. The clinical supervisor or clinical director will approve or deny the tracking form in TA and provide a brief overview of why the

decrease in services was approved or denied. Below is a general description of each treatment phase and key risk factors it addresses.

TREATMENT PHASE DETAILS

PHASE 1: PREPARATORY PHASE OF TREATMENT

HOPE's Phase 1: Preparatory Treatment Manual https://drive.google.com/file/d/1JxPftxAkzrjO9n6jp9omJPQxJJEP5IFh/view?usp=sharing consists of 6 Modules designed for male and female individuals who have been convicted of a sexual offense, who have engaged in inappropriate or compulsive sexual behaviors, and/or who have realized that they need help in addressing their behavior(s). The Prep Phase consists of the following:

- Roughly 24 open/rolling sessions (12-weeks) utilizing cognitive-behavioral treatment for adult males who have been convicted of a sexual offense.
- Emphasis is placed on motivation and engagement as the primary means to introduce and engage offenders in treatment.
- Group facilitator should adopt a reinforcing and encouraging approach, while also being firm and setting boundaries.
- In this phase, it is essential that participants understand how this treatment program can benefit them and help them achieve their life goals.
- This phase has two goals:
    - 1) Providing individuals with the knowledge of HOPE Program policy and procedures, as well as relevant treatment concepts that will prepare them for success in subsequent treatment phases and within the community.
    - 2) Motivating and engaging clients, establishing appropriate group climate, and beginning the development of an effective therapeutic relationship.

After successful completion of the 12-week preparatory phase of treatment, individuals will be assessed for transition to HOPE's core phase of treatment.

PHASE 2: CORE PHASE OF TREATMENT

- The Core Phase of treatment is split into two Blocks; Block 1: Emotional and Cognitive Self-Regulation Manual

https://drive.google.com/file/d/1JtfUCSHk1yUxUIRJ2O7IUhIApY1ES7PH/view?usp=sharing and Block 2: Interpersonal Relationships and Sexual Self-Regulation Manual https://drive.google.com/file/d/1K8Nc1PjcrZGbYRZj8nzAGeziHr6VYNV3/view?usp=sharing, totaling 24 models. In Phase 2, our clients actively address personal dynamic risk factors that lead to sexually abusive behavior(s). Listed below are the key objectives of this treatment phase designed to help clients advance to the next stage:

- Reduction and/or management of the offender's deviant sexual urges and recurrent deviant fantasies.
- Identification and treatment of the offender's thoughts, emotions, and behaviors that facilitate sexual re-offenses or other victimizing or violent behaviors.
- Reduction in negative emotionality.
- Understanding of sex drive and preoccupation.
- Decreased level or removal of hostility.
- Reduction in deviant sexual interest and active practicing of healthy sexuality.
- Exploration of victim impact and victim empathy.
- Ability to utilize internal controls.
- Abstinence from all drugs and alcohol.
- Completion of a non-deceptive polygraph, to include the absence of disclosures.
- Participation in the Chaperone Program.
- Offender education (including those identified as the offender's support system) about the potential for re-offending and the offender's specific risk factors.
- Self-management methods to avoid a sexual re-offense.
- Demonstrated understanding about non-abusive, adaptive, legal, and pro-social sexual behaviors.
- Demonstrated understanding about the impact of sexual offending on victims, their families, and the community.
- Identification and treatment of the effects of trauma and past victimizations on offenders as factors in their potential for re-offending (it is essential that offenders be prevented from assuming a victim stance as a mechanism to diminish responsibility for their actions).
- No evidence of sexual preoccupation.

- Demonstrated ability of healthy sexual outlets.
- Capacity for relationship stability.
- Successful completion of family reunification requirements (when applicable).
- Stable housing, employment, and financial responsibility.
- Abstinence from all drugs and alcohol.
- Full compliance with the HOPE Program's Treatment Service Agreement.
- Full compliance with Parole/Probation/Court Supervision.
- Healthy Sexuality

Length of the CORE Phases of treatment are dependent upon the offenders' compliance, participation, and completion of homework assignments. Clients categorized as HRSO must complete all modules and homework in the Core Treatment Phase twice; once in an HRSO group and once in a Non-HRSO group (see titration requirements). Clients identified as Non-HRSO must complete all modules and homework in the Core Treatment Phase once.

PHASE 3: MAINTENANCE PHASE OF TREATMENT

Clients who enter Phase 3: Maintenance Program Manual https://drive.google.com/file/d/1M22Ayt7rj1GUSyEl9pB-2c1o_jRt4x-J/view?usp=sharing have demonstrated an ability to successfully apply the skills learned during treatment Phases 1 and 2 to their everyday lives. The Maintenance Phase focuses on providing continued support and reinforcement through ongoing structure to help clients properly internalize what they have learned. Key elements are as follows:

- Bi-monthly group psychotherapy attendance.
- Monthly individual psychotherapy attendance.
- Continued abstinence from all drugs and alcohol (AA/NA attendance when required).
- Maintenance of a prosocial peer group.
- Adherence to the Good Lives Model.
- Adherence to the Relapse Prevention Model.
- Completion of a non-deceptive polygraph, to include the absence of disclosures.
- Stable housing, employment, and financial responsibility.

- Maintenance of treatment gains.
- Full compliance with Parole/Probation/Court Supervision.
- Discharge Planning.

It is important to notes that clients can return to the CORE Phase of treatment should their risk levels increase, should they violate their conditions of probation or parole, or should they fall below treatment expectations in this phase of the program.

PHASE 4: AFTERCARE PHASE OF TREATMENT

Clients who have completed the Maintenance Phase of treatment should be placed in HOPE's Aftercare Program. The Aftercare Phase of treatment is designed to comply with state law requiring registered 290 offenders to maintain their connection with treatment for the duration of their term on community supervision. Key elements are as follows:

- Occasional individual treatment sessions (a minimum of once per quarter)
- Twice yearly polygraph appointments
- Adherence to the Good Lives Model
- Adherence to the Relapse Prevention Model
- Review of key concepts
- Maintenance of treatment gains

CRITERIA FOR GRADUATION, RELEASE, AND/OR TERMINATION FROM TREATMENT

Clients discontinue treatment at HOPE for various reasons. Due to specific guidelines within our contracts, HOPE is prohibited from discharging a client from treatment for any reason; however, HOPE has a zero-tolerance policy against violence and threatening behavior. If any client engages in violent or threatening behavior at HOPE, the police and supervising officer are to be contacted, immediately.

Grounds for termination or release from treatment include rearrests for violating ones' general or special conditions of parole or probation, rearrests for committing a new offense(s), receiving violations for failure to regularly report for treatment, failure to comply with program policies, moving out of the county, and no longer being mandated to attend treatment. Before discharging

any client from TA, the clinician and or operations is to confirm the discharge date and reason for discharge and document in collateral.

In order to graduate from HOPE Program, a client must satisfy several criteria. Graduation criteria is as follows:

1. The client must successfully complete the Maintenance Program.
2. The client must continue to adhere to all HOPE titration criteria
    a. Appropriate behavior in group and in the community (i.e., demonstration of an ability to apply group content to real world situations).
    b. Measurable and sustained decrease in risk to re-offend
    c. Completion of a non-deceptive maintenance polygraph (including disclosures) that targets areas of risk for the client (unauthorized contact with minors, use of pornography, etc.).
    d. Full compliance with conditions of community supervision
3. Clinician must complete the CASOMB titration tracking form in TA and provide a detailed presentation to staff in group supervision or team meeting.
4. A team decision is made regarding suitability for graduation. If the client is considered appropriate for graduation, he can transition to Phase 4 (Aftercare), which consists of individual-only, supportive treatment.
5. The Clinical Director (CD) or supervisor must approve or deny the CASOMB titration form in TA and include their reasoning for final decision.
6. The clinician should also complete the HOPE Treatment Completion Certificate. Operations will upload this certificate into ARMS. Clinicians are **not permitted** to provide this certificate directly to clients. Clients can reach out via parole to obtain this documentation if they wish.

See special populations for specific criteria to be terminated from treatment.

SPECIALIZED PROGRAMING

CHAPERONE PROGRAM

HOPE Program has developed a Chaperone Program designed to educate the potential Chaperone on the overall dynamics of sex offenders and covers important topics including the definition of sexual abuse, the difference between a person who molests a child and a pedophile, grooming, and other topics related to sexual re-offense. HOPE's Chaperone Program is designed to be an additional barrier to sexual offending.

Who is Chaperone Training for?

- Primary caregivers of adolescent sexual abusers and children with sexual behavior problems.
- Foster care providers with children who may have acted out sexually in the past.
- Non-offending parents/caregivers who might need to chaperone a sexual abuser during a specific event, such as church, a wedding, funerals, etc.
- Significant others, friends, family members, and/or co-workers of a sexual offender who want to assist that person in remaining offense free.

Chaperone Requirements:

- Must be 21 years of age or older.
- Must have no prior accusation, charge, and/or conviction regarding sexual abuse.
- Must be able to cognitively understand the training materials.
- Must acknowledge, in writing, that they believe the sexual offense occurred (i.e., not in denial or colluding with the client).
- Must sign the required participant documents.
- Must be substance free.
- Must report suspected child abuse to appropriate authorities.

Why do Chaperone training?

- Sex Offenders do not live in a vacuum. Most persons who have committed a sexual offense are living in the community.

- It is not unusual for incest victims to want contact with their perpetrator (i.e., father, brother, sister, etc.).
- At some point in the future, the offender will not be supervised; therefore, the information and training provided to Chaperones will be, in the future, the last line of defense.
- Chaperone Training is not defined by a limited number of sessions, but a process of education and understanding to cultivate awareness and minimize re-offense. It is not family reunification. It is not in lieu of sexual offender treatment, but meant to compliment it.

The Goals of Chaperone Treatment are:

- Prevent further victimization.
- Assist offenders in controlling their inappropriate and/or sexually offending behavior.
- Provide information and tactics for effective chaperoning of sex offenders.
- To Allow offenders a safe way to participate in events.
- Increase awareness of dynamics and scope of sexual abuse.
- Provide more accountability for the offender.

Chaperone Timeline-Example:

Phase 1

Sessions 1& 2: First two sessions are used to interview chaperone for appropriateness as a chaperone and develop rapport.

Sessions 3 through 8: Multiple sessions reviewing the dynamics of sexual offending, the difference between a person who molests a child and a pedophile, grooming behaviors, high risk situations, common problems related to chaperoning an offender.

At the conclusion of sessions 1 through 8, the Chaperone will take a test to ensure understanding of the material. The Chaperone must answer all questions correctly to move on to Phase 2.

Phase 2

Session 9: Chaperone and client attend together. Brief review of Phase 1. Review mandatory reporting laws, no alcohol/drug policy, no child left alone policy. Review of Phase 2 slides. Clinician assigns the client the Account of Offense assignment

Session 10: Chaperone and client attend together. Client presents Account of Offense with time to review and process the details. Clinician assigns the client the Offense Chain assignment

Session 11: Chaperone and client attend together. Client presents Offense Chain assignment with time to review and process details. Clinician assigns the My Good Life Plan Assignment.

Session 12: Chaperone and client attend together. Client presents the My Good Life Plan assignment with time to review and process details. Final review and questions.

TELETHERAPY

In some cases, HOPE may conduct therapy over the telephone or via HOPE's remote desktop stations with approval from stakeholders if clients are physically unable to attend in-person sessions for reasons such as limited mobility or a physical impairment that could lead to injury, or if required travel time is over 80 minutes, one way, to the nearest HOPE clinic.

TELEHEALTH PROCEDURE

Before the delivery of health care via telehealth, the health care provider initiating the use of teletherapy shall inform the client about the use of telehealth and obtain verbal or written consent from the client for the use of telehealth as an acceptable mode of delivering health care services and public health. The consent shall be documented.

Upon initiation of telehealth services, a clinician shall do the following:

1. Obtain informed consent from the client consistent with Section 2290.5 of the Code of the BBS.
2. Inform the client of the potential risks and limitations of receiving treatment via telehealth.
3. Provide the client with his or her license or registration number and the type of license or registration.
4. Document reasonable efforts made to ascertain the contact information of relevant resources, including emergency services, in the client's geographic area.

Each time a clinician provides services via telehealth, they shall do the following:

1. Assess whether the client is appropriate for telehealth, including, but not limited to, consideration of the client's psychosocial situation.
2. Utilize industry best practices for telehealth to ensure both client confidentiality and the security of the communication medium.

The process of explaining and obtaining informed consent, by whatever means obtained, sets the stage for the relationship between the clinician and the client. Clinicians make reasonable effort to offer a complete and clear description of the telehealth services they provide and seek to obtain and document informed consent when providing professional services (APA Ethics Code, Standard 3.10). In addition, they attempt to develop and share the policies and procedures that will explain to their clients how they will interact with them using the specific telecommunication technologies involved. It may be more difficult to obtain and document informed consent in situations where clinicians provide telehealth services to their clients who are not in the same physical location, or with whom they do not have in-person interactions. Moreover, there may be differences with respect to informed consent between the laws and regulations in the jurisdictions where a clinician who is providing telehealth services is located, and the jurisdiction in which the client resides. Furthermore, clinicians may need to be aware of the manner in which cultural, linguistic, socioeconomic characteristics, and organizational considerations may impact a client's understanding of, and the special considerations required for, obtaining informed consent (such as when securing informed consent remotely from a parent/guardian when providing telehealth services to a minor).

Telehealth services may require different considerations for and safeguards against potential risks to confidentiality, information security, and comparability of traditional in-person services. Clinicians are thus encouraged to consider appropriate policies and procedures to address the potential threats to the security of client data and information when using specific telecommunication technologies, and they are to appropriately inform their clients about them. For example, clinicians who provide telehealth services consider addressing with their client's what client data and information will be stored, how the data and information will be stored, how it will be accessed, how secure is the information communicated using a given technology, and

any technology-related vulnerability to confidentiality and security by creating and storing electronic client/patient data and information.

TELEHEALTH SESSION RULES

1. Clinic session rules still apply
2. Clients must call in on time (15-minute grace period for group and 10-minute grace period for individual sessions). If they call in 1 minute past the grace period, they need to be dismissed from the session, similar to if they were to arrive late to an in-person session.
3. Clients must be in a confidential location, not distracted, prepared with their homework assignments, and in a space where they can verbally participate.
4. Clients should respond when called upon so the clinician can give the client credit for their active participation if sessions are scheduled via telephone and not video.
5. Clients must be in a stationary location and cannot be driving, riding the bus, riding their bike, grocery shopping, working, cooking, etc. Keeping structure has been difficult and is a slippery slope.
6. In the progress note, indicate that this was a teletherapy session by stating that the session was conducted via phone (teletherapy). You can say things like, "Mr. (insert client name) reported that he was feeling _____ because of _____." This still keeps some of the face-to-face structure.
7. California Department of Corrections and Rehabilitation (CDCR) no longer requires paper sign-in sheets and requires electronic documentation only. Probation does continue to require paper sign-in sheets for both remote and in-person sessions. For instructions on documenting attendance for CDCR and Probation clients, see *Telehealth Guidance* section below or follow the link: https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/telehealth-guidance

TYPICAL TELETHERAPY SESSIONS LOOK SOMETHING LIKE THIS:

1. The client calls into their scheduled session on time (10-minute grace period; same as in-person sessions).

2. If CDCR, the client gives their physical address so the clinician can document it in TreatAnyone (TA). For instructions on this, see https://sites.google.com/a/almaxconsulting.com/hope/frequently-asked-questions/telehealth-guidance/process-how-do-i-schedule-and-mark-attendance-for-teletherapy-clients. If Probation, clinician writes "teletherapy" in both the Sign-in and Sign-out boxes.
3. The clinician has the client engage in a brief check-in.
4. The clinician turns to the last lesson for a brief review of material.
5. The clinician engages the client in discussion on the curriculum topic and gives lots of reality-based examples of abstract principles.
6. The client presents their homework, if applicable.
7. The clinician assigns the client homework, if applicable.
8. The clinician confirms the client's next scheduled session (time/day, etc.).
9. The clinician checks in to confirm the client's ending address location so it can be documented in TA; see link above for further instruction.
10. Check out; clinician marks attendance and completes Progress Note (PN) in TA.

TELEHEALTH DOCUMENTATION

The California Department of Corrections and Rehabilitation (CDCR) requires contractors to enter the start and end addresses of teletherapy clients into their ARMS database; therefore, all teletherapy clients must now be scheduled with the Individual-Teletherapy offering and an Attendance Note created with the start and end addresses in TreatAnyone (TA). The client's physical address location must be entered for both group and individual teletherapy sessions.

CDCR Overview

1. If the client remains in the same location at the start and end of the call, only enter that one address. You do not need to include the apartment number, state, or zip code. Please include the street number and street name. Cross streets are also accepted (this may apply to transient clients).
2. If the client ends the call at a different address than at the start of the call, enter both addresses. You do not need to include the apartment number, state, or zip

code. Please include the street number and street name. Cross streets are also accepted (this may apply to transient clients).

Example:

1. 6836 Stonewall Court, Stockton
2. Walgreens on Hammer Lane, SF
3. 1401 Telegraph/8974 A Street, Salinas

Individual Telehealth Sessions:

1. Schedule as Individual-Teletherapy appointment in TreatAnyone (TA)
2. Mark attendance as present
3. Click the Green Attendance Note icon (at the far right of the attendance listing) and enter the Starting and Ending Addresses as seen in the examples above
4. Click Save

Addresses are not required for No Shows and Excused appointments.

Group Telehealth Sessions:

1. Schedule Groups as normal (we currently do not have a separate group teletherapy offering, but the group title should be labeled as telehealth in TA)
2. Mark attendance as present
3. Click the Green Attendance Note icon (at the far right of the attendance listing) and enter the Starting and Ending Addresses - Cross streets are acceptable (See examples above)
4. Click Save

Addresses are not required for No Shows and Excused appointments

<u>County and Federal Probation</u>

Individual and Group Telehealth Sessions

1. Schedule as Individual or Group (we do not currently have separate teletherapy offerings).
2. Mark attendance as present.
3. Click the Green Attendance Note icon (at the far right of the attendance listing) and write "Telehealth."

4. Click Save.

Addresses are not required for Federal and Probation Clients. Sign-in sheets are still required and need to say "Telehealth" in the Signature In and Signature Out boxes, respectively. No show and Excused will remain the same.

APA Guidelines for Telepsychology- Psychologist Competency

1. Be up to date on latest research regarding remote/telepsychology "…Psychologists assume responsibility to continually assess both their professional and technical competence when providing telepsychology services. Psychologists who utilize or intend to utilize telecommunication technologies when delivering services to clients/patients strive to obtain relevant professional training to develop their requisite knowledge and skills."
2. Goodness of fit "… Psychologists are encouraged to examine the available evidence to determine whether specific telecommunication technologies are suitable for a client/patient, based on the current literature available, current outcomes research, best practice guidance and client/patient preference…" and "…Psychologists understand the need to consider their competence in utilizing telepsychology as well as their clients/patients ability to engage in and fully understand the risks and benefits of the proposed intervention utilizing specific technologies…."
3. Document rationale for use of telepsychology "…Additionally, psychologists are encouraged to document their consideration and choices regarding the use of telecommunication technologies used in service delivery…"
4. Emergency protocols… "Psychologists who are trained to handle emergency situations in providing traditional in-person clinical services, and are generally familiar with the resources available in their local community to assist clients/patients with crisis intervention. At the onset of the delivery of telepsychology services, psychologists make reasonable effort to identify and learn how to access relevant and appropriate emergency resources in the client's/patient's local area, such as emergency response contacts (e.g., emergency telephone numbers, hospital admissions, local referral resources, clinical

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champion at a partner clinic where services are delivered, a support person in the client's/patient's life when available). Psychologists prepare a plan to address any lack of appropriate resources, particularly those necessary in an emergency, and other relevant factors which may impact the efficacy and safety of said service. Psychologists make reasonable effort to discuss with and provide all clients/patients with clear written instructions as to what to do in an emergency (e.g., where there is a suicide risk). As part of emergency planning, psychologists are encouraged to acquire knowledge of the laws and rules of the jurisdiction in which the client/patient resides and the differences from those in the psychologist's jurisdiction, as well as document all their emergency planning efforts…"

5. Discharge or Referral "…as applicable psychologists are mindful of the array of potential discharge plans for clients/patients when telepsychology services are no longer necessary and/or desirable. If a client/patient recurrently experiences crises/emergencies suggestive that in-person services may be appropriate, psychologists take reasonable steps to refer a client/patient to a local mental health resource or begin providing in-person services…"

OVERVIEW OF THE PROCESS FROM REFERRAL TO SUCCESSFUL TREATMENT COMPLETION

Before entering treatment, all clients are to go through our referral and evaluation process to determine the appropriate treatment assignment, as discussed above. Figure 4 below provides a general overview of the process from entering treatment to concluding treatment. Specifically, it illustrates the specific steps and activities associated with all new referrals and an overview of the clients' experience when initiating treatment at HOPE. Figure 4 also illustrates the individualization of HOPE when determining the client's treatment frequency. For instance, under "Individual & Group Assignment," treatment dosage is determined by taking into consideration the referring agency and the outcome of the evaluation. Clients are assigned accordingly to the appropriate individual and group needs. Clients can be seen for weekly, bi-weekly, monthly or quarterly individual treatment; weekly group, bi-weekly group, or no group. See Figure 4 below for an overview of the process from referral to treatment completion.

General overview of the process from entering treatment to concluding treatment at the HOPE Program.

Figure 4 Treatment Process

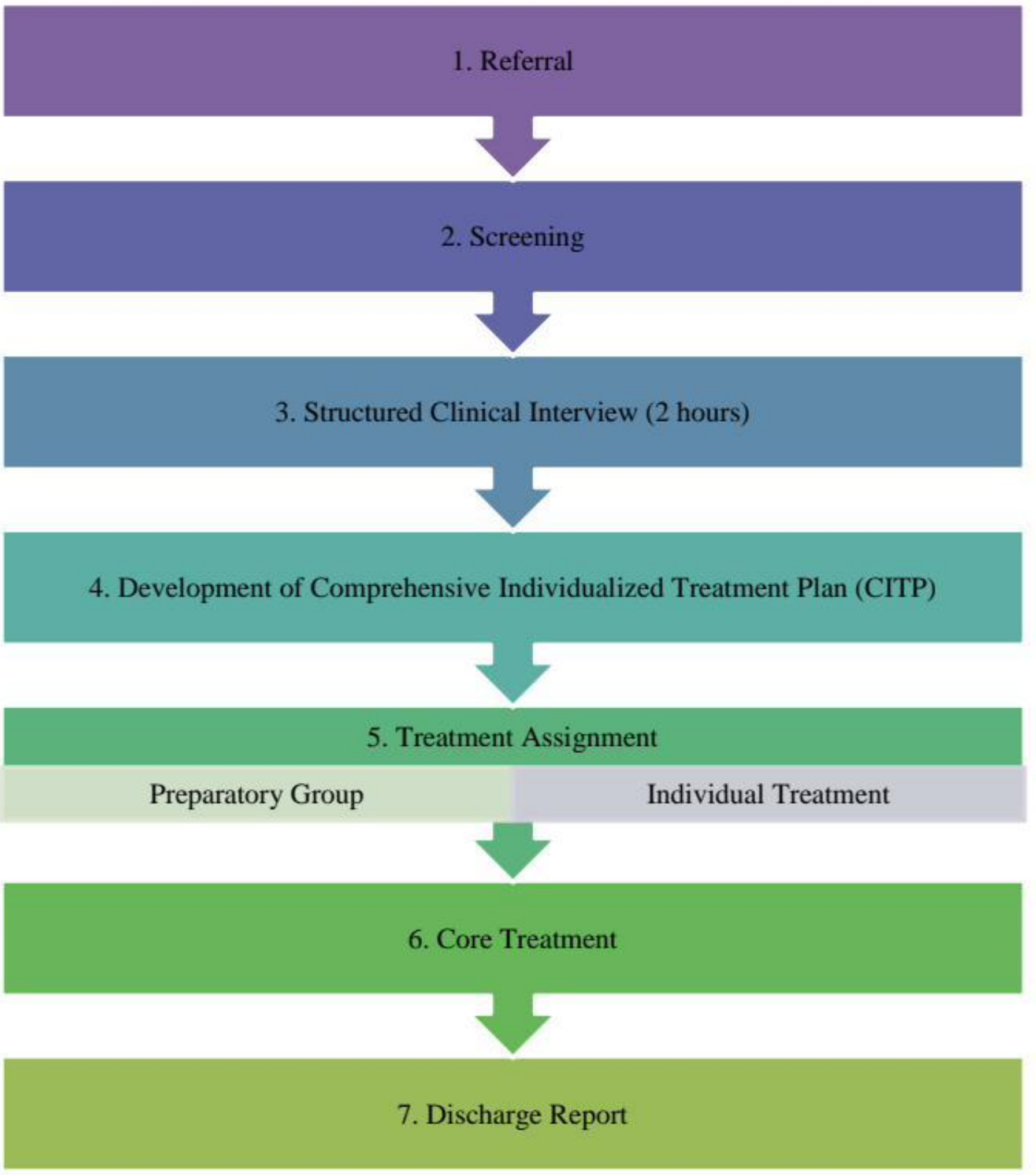


Figure 4.1 Detailed Overview of Referral to Treatment

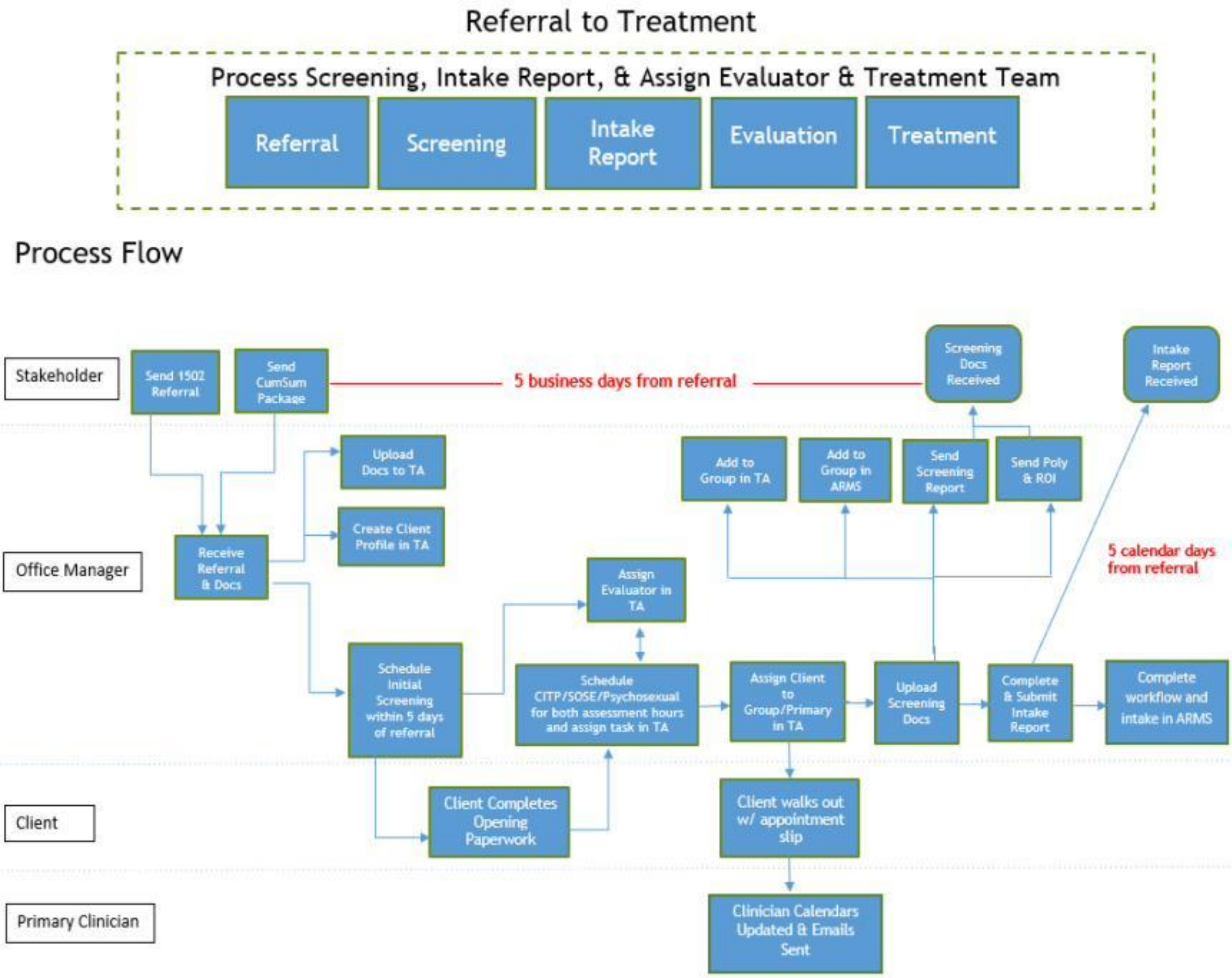
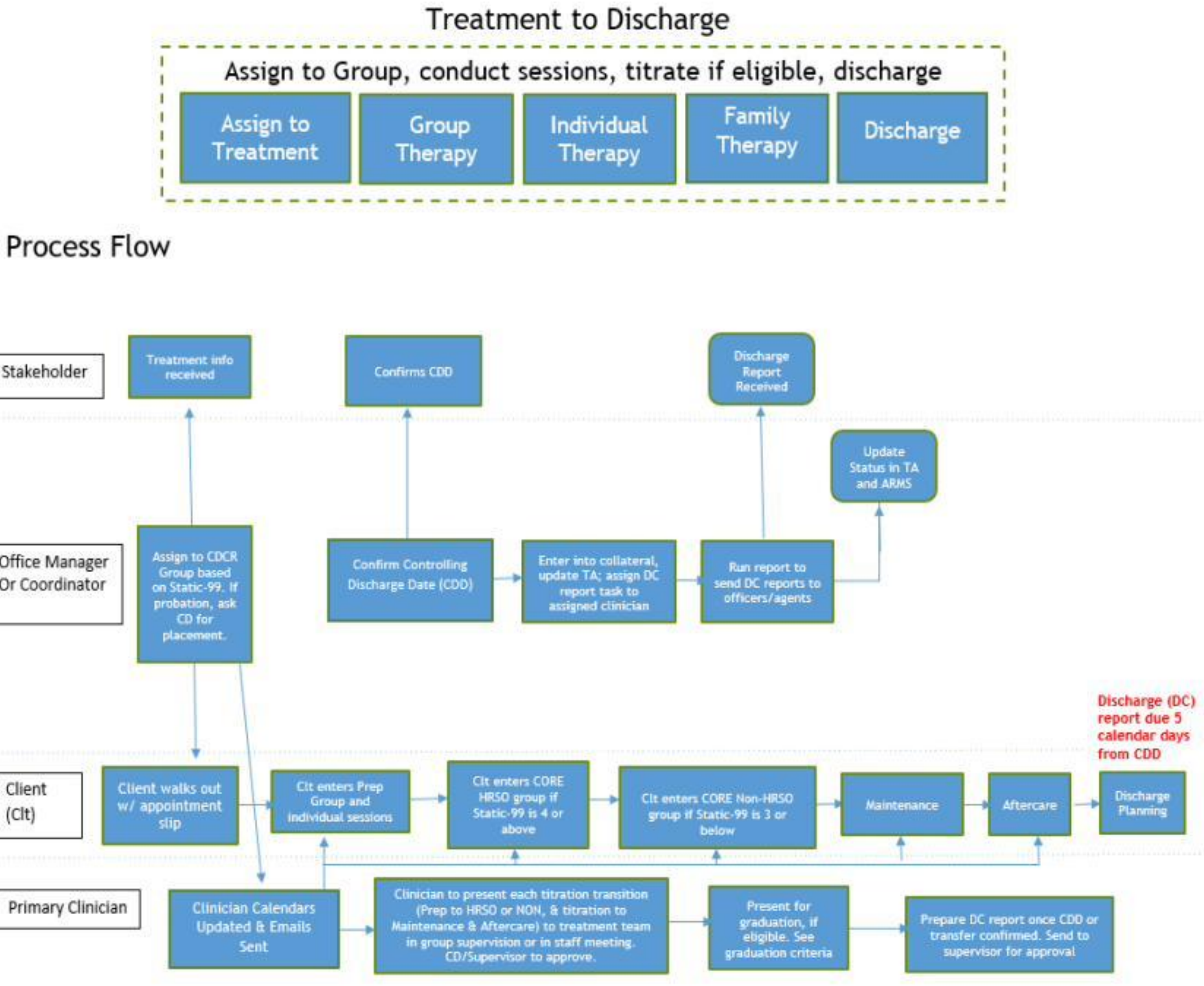


Figure 4.2 Detailed Overview of Treatment to Treatment Completion



ONGOING ACTIVITIES WHILE ENROLLED IN HOPE

Figure 5 & 6 below illustrates the ongoing activities associated with client populations when participating in treatment.

Figure 5 Ongoing Activities while enrolled in HOPE

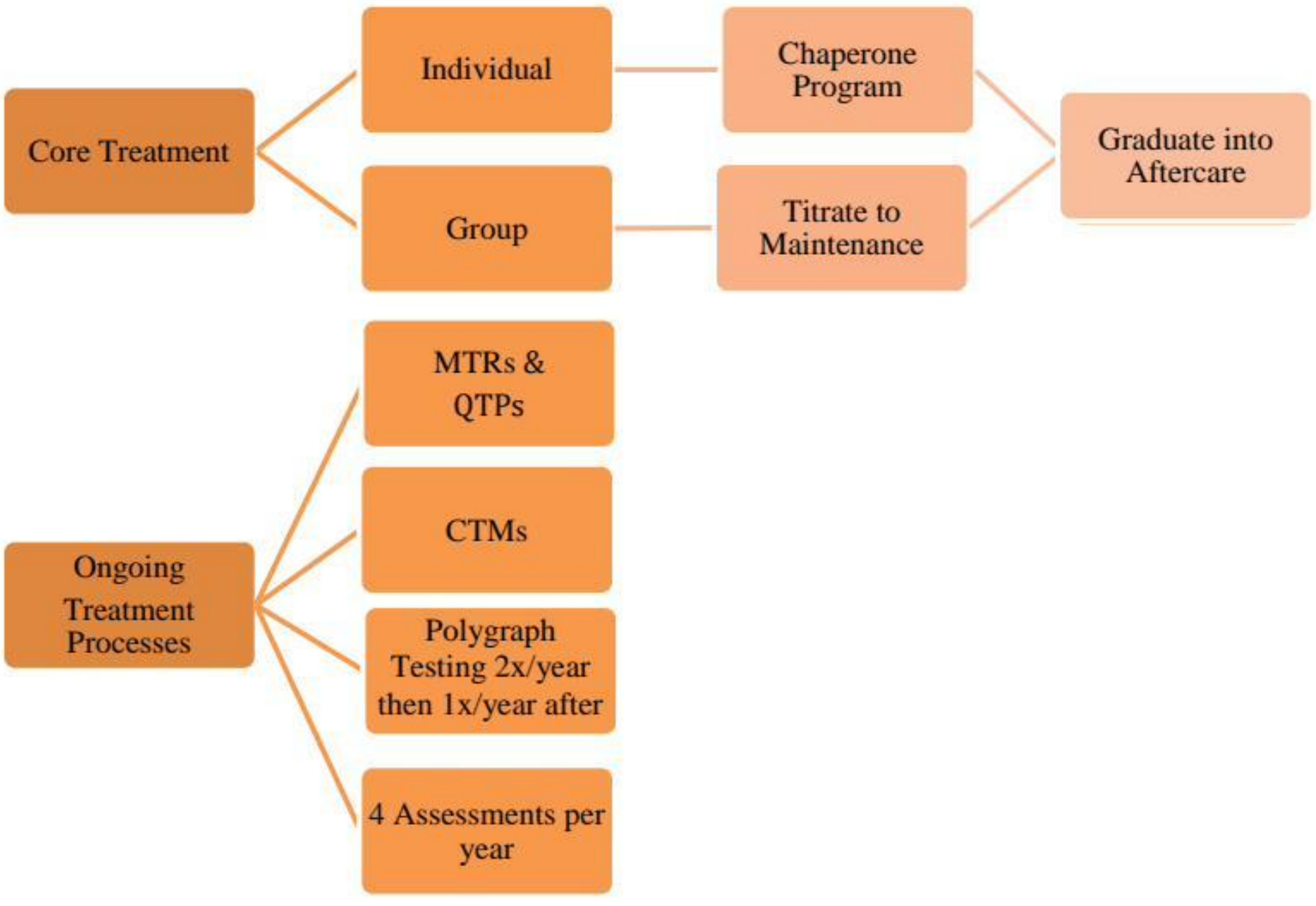
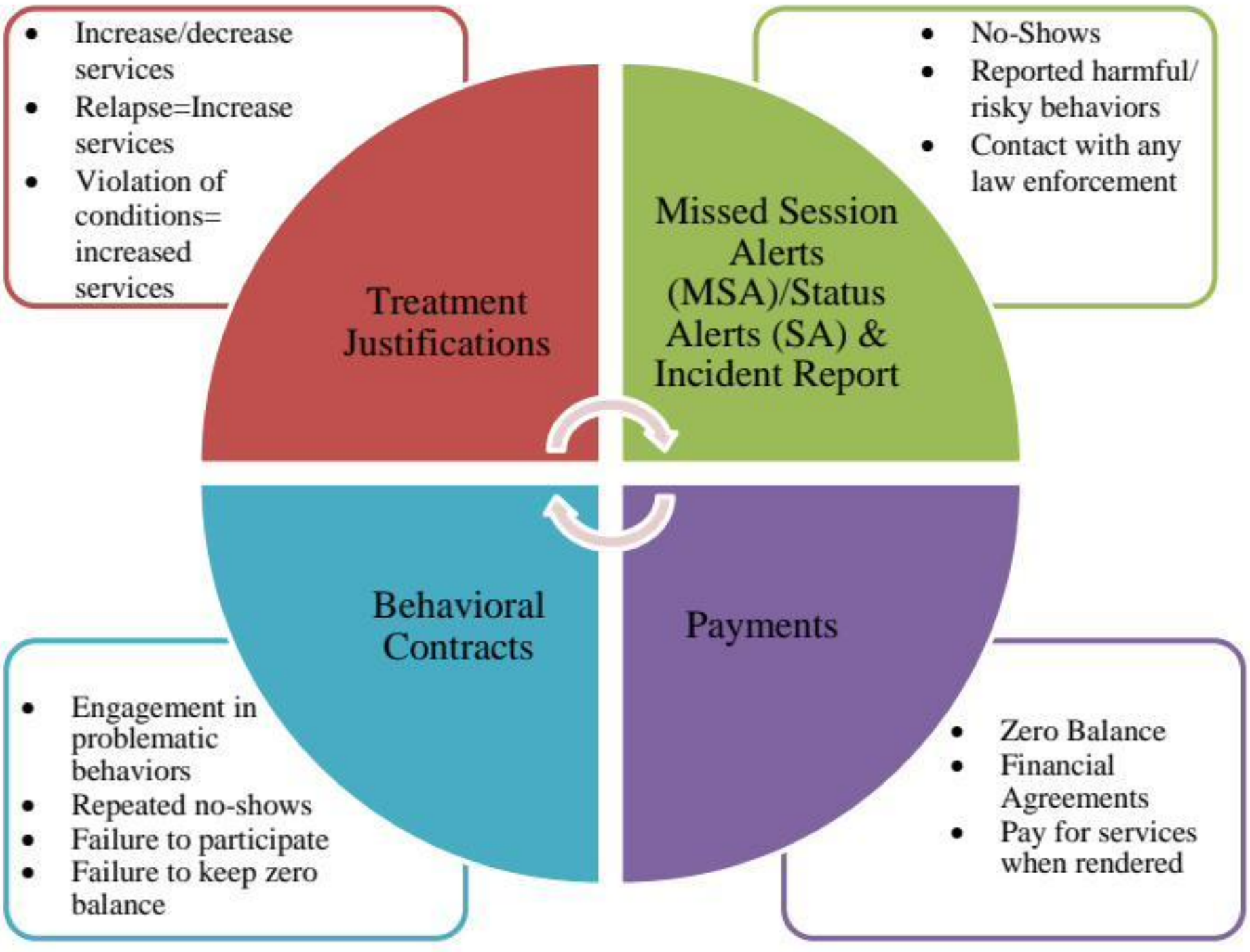


Figure 6 Ongoing Activities while enrolled - Micro View



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Referral Process

Once a referral is received by HOPE, operations will:

- Create a new client file in TA.
- Collect necessary documentation for the referral.
- Schedule the Initial Screening.
- Schedule the Comprehensive Individualized Treatment Plan (CITP), Sex Offender Specific Evaluation (SOSE), Psychosexual Evaluation, Substance Abuse (SA) Assessment, or Mental Health (MH) Assessment with the appropriate clinician, which is dependent on referral source.

Therapeutic Contract and Informed Consent with Clients

- During the intake process, all clients are provided with a treatment contract and informed consent.
- Clients are not allowed to participate in any services at HOPE until all the paperwork has been signed and the client has provided consent to engage in mental health treatment.

Handling of Confidential File Information, Release Forms

HOPE Program takes all reasonable steps to protect the confidential records of our clients and private nature of the services we provide. HOPE utilizes an electronic client file system, known as TreatAnyone (TA). We require a written, signed client consent to disclose the defendant's/offender's records. CDCR and Federal clients are not provided with any documentation other than their proof of practice slip, as CDCR and/or the Federal Government is the primary client.

Policies on the Use of File Information for Research

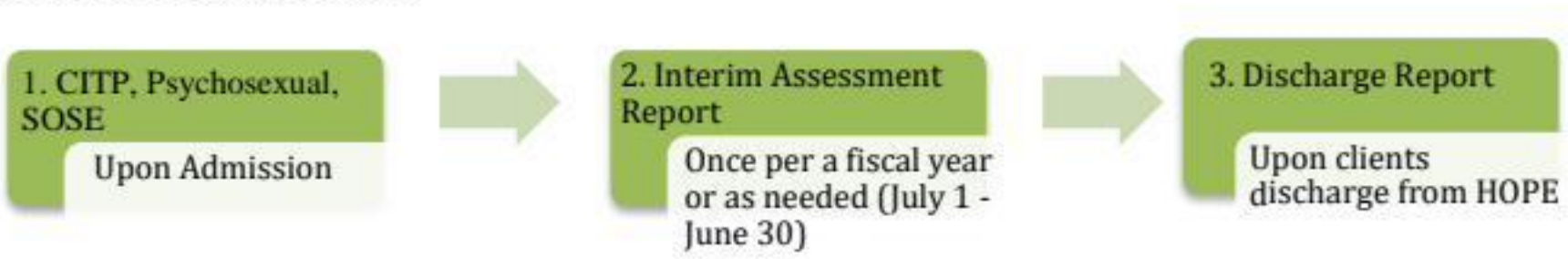
HOPE Program requires signed client consent for research participation. Information and data obtained from psychological and psychophysiological tests, as well as any actuarial measures completed, may be used for research regarding the understanding of sexual behavior. The information used for research purposes will be de-identified.

Comprehensive assessments & Reports

HOPE has implemented comprehensive assessment batteries intended for all clients. The purpose of these assessments is to help guide clinical decisions, identify risk-relevant treatment needs, and demonstrate the overall effectiveness of our interventions. Assessments are utilized throughout the duration of an individual's treatment at HOPE.

Three main types of reports are utilized over the course of treatment. If a report is submitted and additional reports that year are required, clinicians can utilize interim assessment reports.

Types of Reports



Comprehensive Individualized Treatment Plans (CITPs)

A crucial first step in providing treatment is conducting a Comprehensive Individual Treatment Plan (CITP) and risk assessment that includes assessments based on historical factors, or static risks, and some that are based on long-term vulnerabilities or dynamic risks. Prior to starting treatment, every client referred to HOPE undergoes an evaluation either referred to a Sex Offender Specific Evaluation or commonly known as a CITP. The content and measures used in the assessment are determined by the reason for referral and referring organization.

CITPs give an initial understanding of each new client. The CITP consists of a structured clinical interview with the client and the completion of a combination of SARATSO and HOPE selected risk assessment tools. A structured clinical interview is an evaluation conducted by a trained clinician leveraging a variety of assessment methods such as the Level of Service Case Management Inventory (LS/CMI), STABLE-2007, The LOOK, The Sex Offender Treatment Readiness Scale (SOTRS), and the Criminal Sentiment's Scale-Modified (CSS-M), which depends on the presenting problem and referring agency. Additional testing can be employed, if

necessary, and this decision depends on the client's specific treatment needs and the clinical judgment of the treating clinician and their supervisor.

Information from assessments is captured in the Comprehensive Individualized Treatment Plan (CITP), which among other things, identifies treatment needs and interventions for each individual. The treatment plan for each client is carefully tailored, following the Risk, Need, Responsivity Principle, to address specific needs identified during the intake and initial evaluation process.

A CITP will also incorporate Individual treatment goals which serve as the framework for the overall treatment plan. Individual treatment goals are defined when a mental health needs assessment and a comprehensive sex offender-specific risk assessment are conducted with each client. The goals focus on addressing personal skill deficits, predilections, and learned behaviors that highly correlate with sexual recidivism. Each CITP will include a minimum of three individual treatment goals; one sexual (obtained from the STABLE-2007), one violent (obtained from the LS/CMI) and one that is a general criminogenic treatment need.

A COMPREHENSIVE EVALUATION INCLUDES THE FOLLOWING INFORMATION				
Functioning	Intellectual & Cognitive Functioning	Neuropsychological Functioning	Academic Achievement	
Mental Health	Character/ Personality Pathology	Mental Illness	Self-Concept/Self Esteem	Psychological Adjustment
Medical Psychiatric Physical Health	Pharmacological Needs	Medical Condition	History of Medication Use/Abuse	
Employment Recreation Leisure	Use/Abuse & Number of Relapses	Number of times employed/fired/quit without giving notice	Activities in Spare Time, Hobbies, Interests	
Drug/Alcohol Use	Preferred Drugs Used (legal and illegal)	Frequency and Amount of Use/Abuse	Rehabilitation, NA/AA groups	
Stability of Functioning	Marital/Family Stability	Employment/ Education	Social Skills	

Developmental History	Sexual History (Onset, Intensity, Duration, Pleasure Derived)	Interpersonal Relationships, Peers/Associates, Intimate Relationships	Reinforcement Structure for Deviant Behavior	Arousal/Interest Pattern	
	Sexual Deviance	Dysfunction (Impotence, Injuries, Medications Affecting Sexual Functioning, Etc.)	Offenders Perception of Sexual Functioning	Impact of Offending behavior	Access to Children
	Preferences (Male/Female; Age; Masturbation, Use of Tools, Food, Current Sexual Practices; Deviant and Non-deviant Behaviors)		Attitudes/ Cognitions	Specifics of Sexual Crime	
Risk of Re-offense	Risk of Failure in Treatment and Supervision	Review of criminal history			
Motivation and Amenability to Treatment	Precontemplative, Contemplative, Preparation, Action				
Polygraph testing	1st recommended polygraph				

STEPS FOR COMPLETING A CITP

1. Prior to the interview:
    a. Read all legal documentation
    b. Determine the appropriate measures and assessments to use
    c. Obtain interview template
    d. Confirm the appointment has been scheduled correctly in TA and identify the amount due at time of scheduled meeting, if applicable
2. Structured clinical interview is scheduled for 2 hours and held in a confidential setting. The clinicians should wait to mark attendance for the second hour as client can still be seen for the second interview hour should they arrive late.
3. Additional testing might be necessary - this is done in collaboration with the supervisor after the interview is completed.

4. All writing of the assessments is to take place in TA[1].
5. Once all narratives and assessments are completed, submit the report to your individual supervisor for review and approval.

STEPS FOR COMPLETING A PSYCHOSEXUAL EVALUATION/ SEX OFFENDER SPECIFIC EVALUATION (SOSE)

1. Prior to the interview:
    a. Read all legal documentation
    b. Determine the appropriate measures and assessments to use
    c. Obtain interview template
    d. Confirm the appointment has been scheduled correctly in TA and identify the amount of money due at time of scheduled meeting, if applicable
2. Structured clinical interview is scheduled for 2 hours and held in a confidential setting. The clinicians should wait to mark attendance for the second hour as client can still be seen for the second interview hour should they arrive late.
3. Additional testing might be necessary - this is done in collaboration with the supervisor after the interview is completed
4. All writing of the assessments is to take place in TA[2].
5. Once all narratives and assessments are completed, submit the report to your individual supervisor for review and approval.

INTERIM ASSESSMENTS & REPORTS

Interim assessments are completed each subsequent fiscal year. The fiscal year is from July 1st to June 30th. They provide updated assessment information and anything else pertinent to the client. Most individuals will have multiple interim assessments reports over the course of treatment.

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[1] All client documentation and reports are to be drafted and saved in TA. Writing and saving in word to transfer "at a later date" is never appropriate.

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